

USER MANUAL · PATIENT HANDBOOK

Kidney Care Companion

Nephro Healthcare Tracker — Personal Edition

Comprehensive User Layout & Patient Data Guide

Version 1.0 | Document Ref: Medical Tracker Manual

For kidney-transplant recipients, their families and their treating nephrologists

About This Manual — Design Note

This manual was designed to feel like the workbook it describes: calm, clear and trustworthy. The same colours that guide you inside the Excel file guide you here, so you will never wonder which sheet a page belongs to.

Colour	What you see	What it means / what to do
Navy	**Deep Navy**	Titles, table headings and banners — the "professional hospital" colour used in every sheet title bar.
Teal	**Clinical Teal**	Section rules, Doctor Pack panels and reassuring "everything is working" highlights.
Orange	**Warm Orange**	Step headings and checklist bars; also the tab colour of the Lab Results and Doctor Visit Log sheets.
Amber	**Caution Amber**	Important notes and "look twice" reminders.
Red	**Alert Red**	Danger boxes and every out-of-limit value — the colour that means "inform your doctor".
Green	**Healthy Green**	Good results, completed checks and the Daily Vitals sheet.
Purple	**Medicine Purple**	The Medication History chapter.

Layout and typography

- **Page:** A4 portrait, generous 2 cm margins and open spacing so that every page can be read comfortably, even on a phone.
- **Type:** one clean sans-serif family. Chapter titles are large and bold; body text is 10.5 pt; screenshot notes are smaller and in italics.
- **Chapters:** each worksheet opens on a new page with a colour band that matches its Excel tab, followed by the same five headings every time (Purpose, Layout, Colour guidance, Data input, Lifecycle).
- **Boxes:** coloured call-out boxes carry the symbols described on the next page. Pictures always have a caption and a short description underneath.
- **Privacy:** every screenshot comes from a real printed workbook, but names, ID numbers, telephone numbers, dates of birth, doctor and hospital details have been blanked out with grey-blue bars.

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Preface

Living with a kidney transplant is a long journey. The good news is that you do not have to walk it alone — and you do not have to rely on memory. Every blood pressure reading, every glass of water, every blood test and every change of medicine tells a small part of the story. When those small parts are written down in one place, your doctor can see the whole picture in minutes.

The **Nephro Healthcare Tracker** is an Excel workbook built for exactly this purpose. You type the readings you already take at home and the results you already receive from the laboratory. The workbook does the rest: it draws the charts, calculates the averages, flags values that are outside safe limits and prepares a one-page summary that your doctor can read quickly.

Who this workbook is for

- **Kidney transplant recipients** who measure blood pressure, pulse, oxygen level (SpO_2), weight, fluids, urine and blood sugar at home.
- **Family members and caregivers** who help to keep records and prepare for hospital visits.
- **Nephrologists and clinic staff** who receive the printed or PDF “Doctor Pack” and want the key facts first, with the daily detail behind them.
- **Anyone managing chronic kidney disease** who wants a structured, easy-to-read health diary (the same sheets work well with small adjustments).

What you gain

- **Early warning.** A rising creatinine, a falling tacrolimus level or a run of high blood-pressure days becomes visible on a chart long before it becomes a surprise.
- **Accurate medicines.** One log shows what you take now, what you stopped, and what changed — and why.
- **Prepared visits.** In a 10–15 minute consultation, the doctor sees a clean summary first. Your questions are not forgotten, and the doctor’s advice is written down before you leave.
- **Peace of mind.** Numbers inside the safe range are shown with a green tick. Numbers outside it are shown in red or orange, so you know when to call.

How it helps you talk with your doctor

Doctors are short of time and patients are often anxious. The workbook bridges that gap. Before each visit you print or e-mail just **three sheets** (about six pages): the **Doctor Referral Report**, the **Dashboard** and the **Daily Vitals Log**. During the visit the doctor writes notes, the illness-management plan, the tests required and the **next visit date** directly on the Referral Report — so nothing is lost between the clinic and your home.

> ⚠ **PLEASE REMEMBER**
>
> This workbook **supports** your care; it does **not replace** medical advice. Red or orange values mean “**inform your doctor**”. Never start, stop or change a medicine on your own because of what a chart shows.

How to Use This Manual

You do not need to read this manual from cover to cover. Start with the Quick-Start in the next chapter, then open the chapter for the sheet you are working on. Each chapter follows the same five-part pattern, so you can find things quickly:

1. **Purpose & Requirement** — why the sheet exists and what it measures.
2. **Visual Layout Blueprint** — a map of the sheet showing where you type and where results appear.
3. **Colour-Coded Guidance** — what each colour on the sheet means.
4. **Data Input Requirements** — the exact fields to fill, with formats.
5. **Lifecycle & Triggers** — when to use the sheet again.

Symbols used in this manual

	**Important / caution**	Something that is easy to get wrong, or that affects your health data.
	**Stop & check**	A step that can damage your records if done incorrectly. Read twice.
	**Tip**	A shortcut or good habit that saves time.
	**Good to know**	Background information that explains why the workbook behaves a certain way.
	**Done when…**	A quick test that tells you the task is complete.
	**Steps**	A short numbered procedure.

Conventions

- **Sheet names** are written in bold, for example **Daily Vitals Log**. They match the coloured tabs at the bottom of Excel.
- **Cell references** such as `B5` or `C4:P4` tell you exactly where to look (column letter first, then row number).
- **Keys** are written like `Ctrl+P`. On a Mac, use `Cmd` instead of `Ctrl`.
- **Dates** are always shown as `dd-mmm-yyyy`, for example `05-Oct-2026`. This format cannot be misread as month-first or day-first.
- **Screenshots** show a real workbook. The numbers in them are examples only; yours will be different.

Abbreviations and Terms

Medical and technical words used in the workbook and in this manual. Normal ranges are shown in the **Lab Reference Ranges** sheet (Chapter 10) and are decided together with your doctor.

Term	Meaning
BP	Blood pressure, in mmHg. Two numbers: systolic (upper) and diastolic (lower).
Systolic / Diastolic	The pressure when the heart beats / rests between beats.
SpO_2	Oxygen saturation of the blood, in %. Measured with a finger pulse-oximeter. Below 94% needs prompt review.
Pulse (bpm)	Heart beats per minute.
Temp (°F)	Body temperature in degrees Fahrenheit. “NT” in the log means “not taken”.
BMI	Body Mass Index = weight (kg) ÷ height (m)². Calculated automatically in the Doctor Visit Log.
FBS	Fasting Blood Sugar — measured before breakfast, in mg/dL.
PPBS	Post-Prandial (after-meal) Blood Sugar.
HbA1c	Average blood-sugar marker over about three months, in %.
PTDM	Post-Transplant Diabetes Mellitus — high sugar that appears after a transplant, often linked to some medicines.
CKD / CKD (T)	Chronic Kidney Disease / CKD of a transplanted kidney.
Tx / Post-Tx	Transplant / after the transplant.
Graft	The transplanted kidney.
Creatinine	Waste product used to judge kidney function. Rising values can be an early warning.
eGFR	Estimated Glomerular Filtration Rate — a calculated measure of kidney filtering, in mL/min.
Tacrolimus (Tac)	An anti-rejection medicine. Its blood level is checked before the morning dose.
Trough / C0	Blood level measured just before the next dose (“C0”).
CNI	Calcineurin inhibitor — a class of anti-rejection medicines that includes tacrolimus.
MPA	Mycophenolic acid — another anti-rejection medicine class.
BKV / CMV	BK virus / Cytomegalovirus — viruses checked after a transplant.
PCR (blood tests)	Polymerase Chain Reaction — a sensitive test that counts virus copies (copies/mL).
PCR / UACR (urine)	Urine Protein- or Albumin-to-Creatinine Ratio, in mg/g. Shows protein leaking into urine.
RE / ME	Routine and Microscopic Examination of urine.
C/S	Culture and Sensitivity — a test for infection.
Hb	Haemoglobin — oxygen-carrying protein in red blood cells.
TLC (WBC)	Total Leucocyte (White Blood Cell) Count.
SGOT/AST, SGPT/ALT	Liver enzymes.
LDL	“Bad” cholesterol.
Oedema	Swelling of feet, ankles or face caused by fluid.
mmHg, mg/dL, mL, kg, mEq/L, U/L, ng/mL	Units: millimetres of mercury, milligrams per decilitre, millilitres, kilograms, milli-equivalents per litre, units per litre, nanograms per millilitre.

Term	Meaning
MRD No.	Medical Record Department number — your hospital file number.
Rx / Presc	Prescription. “Old Presc” and “Rev Presc” in the log mean the old and the revised prescription.
Not Avl	“Not Available” — no value was found for that date.
Dropdown	A small arrow in a cell that opens a list of choices.
Paste Special ▸ Values	A way of pasting numbers only (without formulas). Shortcut: `Ctrl+Alt+V`, then `V`.
Active sheet	The sheet whose tab is highlighted. “Print Active Sheets” prints only the selected tabs.
PDF	Portable Document Format — a read-only file that looks the same on every device.

The Workbook at a Glance

The workbook has **eleven worksheets**, shown as coloured tabs along the bottom of Excel, in this order. Some sheets are for **typing** (yellow cells); some **work themselves out** from what you typed; two are **reference** sheets.

#	Sheet (tab)	Tab	Type	You type?	Doctor Pack	What it holds
1	**How To Print**		Guide	No	—	Printing instructions and checklists.
2	**Patient Info**		Input	Yes (once)	—	Who you are, your doctor and your transplant details.
3	**Daily Vitals Log**		Input	Yes (daily)	★	Daily BP, pulse, SpO_2, weight, fluids, sugar.
4	**Lab Results**		Input	Yes (each test)	—	All blood and urine tests across dates.
5	**Medication History**		Input	Yes (on change)	—	Every medicine: start, stop and changes.
6	**Dashboard**		Automatic	No	★	Alerts, 8 trend charts and the lab snapshot.
7	**Doctor Referral Report**		Automatic + doctor	Notes only	★	One-page summary for the doctor, plus plan and next visit date.
8	**Weekly Summary**		Automatic	No	—	Week-by-week averages and alert days.
9	**Doctor Visit Log**		Readout + record	Yes (each visit)	—	One row per consultation with key values.
10	**Lab Reference Ranges**		Reference	No	—	Normal and post-transplant target ranges.
11	**Dashboard Settings**		Settings	Optional	—	Alert limits that drive colours and tiles.

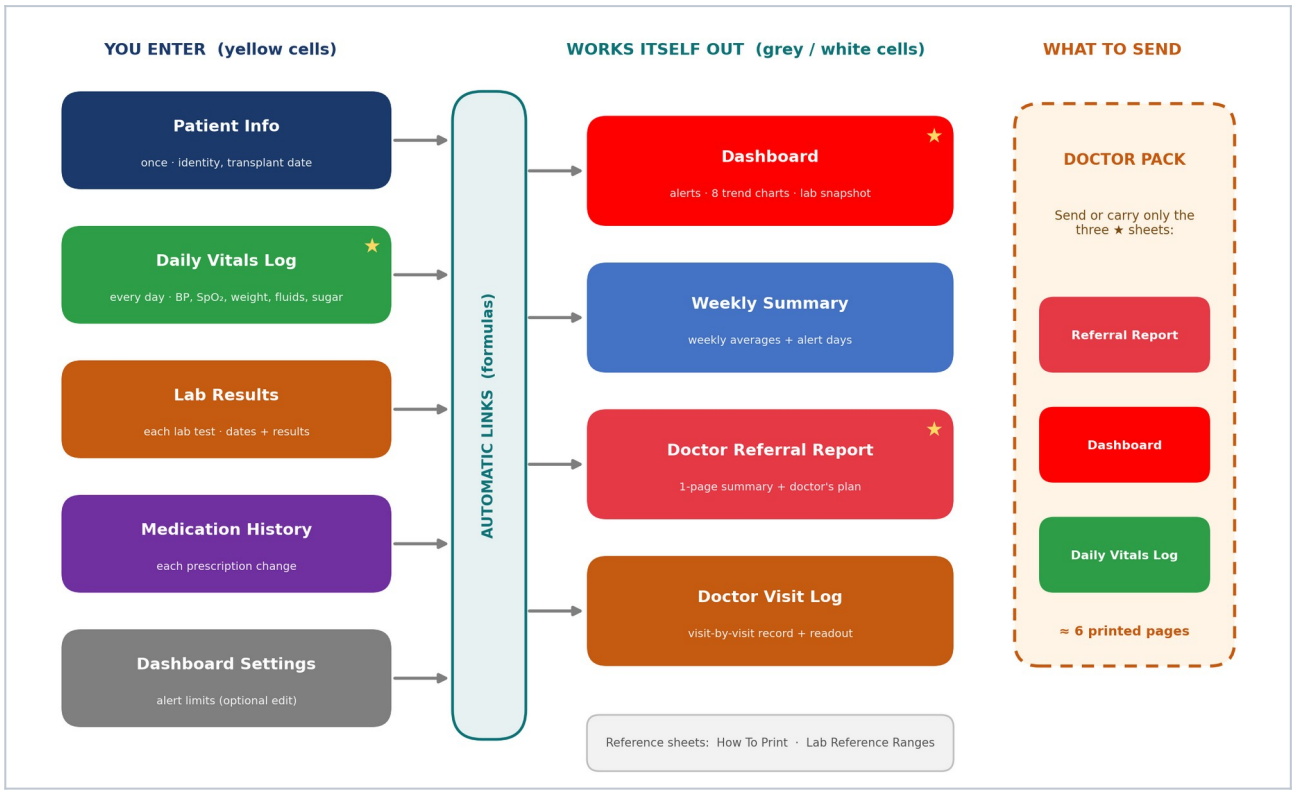


Figure 1 How the sheets are linked

You type into the sheets on the left. Formulas (the central "automatic links" bar) carry that information to the sheets in the middle. Only the three sheets marked ★ go to the doctor. The two reference sheets are for your information.

Which sheet feeds which?

You enter in...	Doctor Referral Report	Weekly Summary	Dashboard	Doctor Visit Log
Patient Info	✔️ identity block, doctor line	—	—	✔️ transplant date (visit schedule)
Daily Vitals Log	✔️ 7-day summary	✔️ weekly averages	✔️ alerts, 4 vitals charts, sugar block	✔️ vitals readout + BMI
Lab Results	✔️ latest results	—	✔️ 3 lab charts, snapshot, alerts	✔️ lab readout
Medication History	✔️ list of active medicines	—	—	—
Dashboard Settings	—	✔️ BP-alert days	✔️ limits, chart lines, tiles	—

Dashboard Settings also set the red/orange colouring and the "alert" flags in the Daily Vitals Log.

Quick-Start: Your First Day

Follow these twelve steps once, and your tracker is ready for daily use. Allow about 30 minutes, with your latest prescription and lab reports beside you.

1. **Make a working copy.** Save the file with a dated name, for example `Nephro_Tracker_05Oct2026.xlsx`. Keep the original untouched as a backup.
2. **Open Patient Info** and fill every yellow cell: doctor, hospital, patient details, **transplant date**, history (Chapter 2).
3. **Open Doctor Visit Log** and type your **height in centimetres** in the yellow HEIGHT box (cell `P2`). Do this once and never change it (Chapter 9).
4. **Open Medication History.** Add one row for each medicine you take now. Set **Status = ACTIVE** and give the start date (Chapter 5).
5. **Open Lab Results.** Type the date of your first test in row 4, then enter each result below it. Add earlier tests too, **oldest first** (Chapter 4).
6. **Open Daily Vitals Log.** From today, type one row per day: date, blood pressure, pulse, SpO_2, weight, fluids, sugar (Chapter 3).
7. **Look at the Dashboard.** Check that the alert tiles and charts have appeared (Chapter 6).
8. **Open Dashboard Settings** and, with your doctor, adjust any alert limit that should be different for you (Chapter 11).
9. **Open Doctor Referral Report** and confirm that your name, medicines and latest labs appear (Chapter 7).
10. **Practise the Doctor Pack.** Select the three ★ sheets and save them as a PDF (Chapter 1). Open the PDF to see what your doctor will see.
11. **Save** the workbook. After every month, also save a dated copy.
12. **Add a daily reminder** on your phone: “Vitals — same time every day”.

✓ **YOUR TRACKER IS READY WHEN**

The **Referral Report** shows your name and medicines, the **Dashboard** shows at least one alert tile with a value, and the **Daily Vitals Log** has today’s row filled in.

Seven golden rules

! **RULES THAT PROTECT YOUR DATA**

1. Type only in **yellow** cells. White and grey cells calculate by themselves.

2. Never rename, move, hide or delete a worksheet.

3. Never insert or delete rows or columns inside a sheet. Use the blank rows already provided.

4. Enter lab dates **oldest first, left to right**, with no empty column between them.

5. When copying numbers from one place to another, use **Paste Special ▸ Values**.

6. Do not sort or filter the logs.

7. Save a dated copy before each doctor’s visit.

CHAPTER 1 · GUIDE SHEET

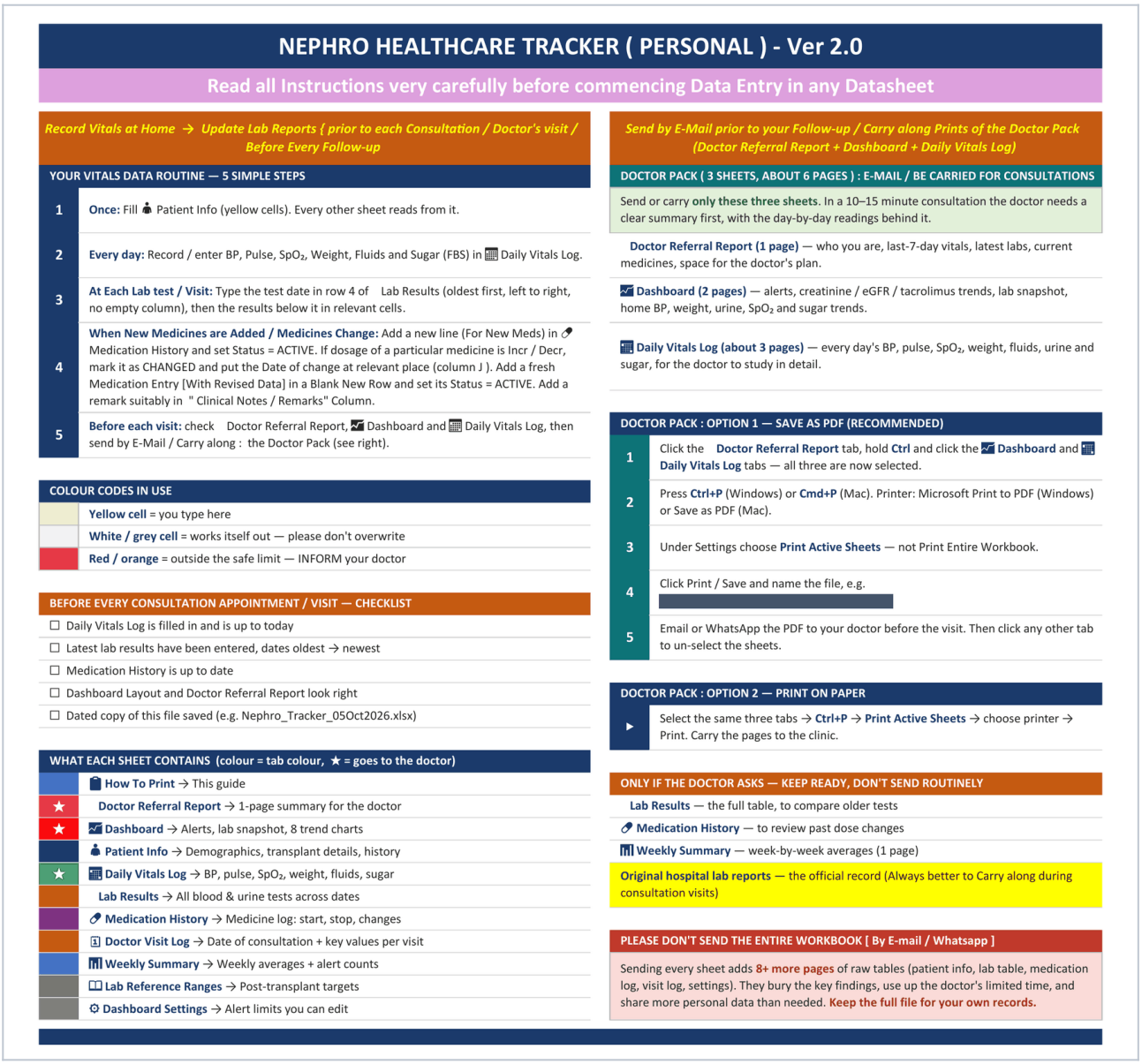
Chapter 1: How To Print

Your guide to the daily routine and to preparing the Doctor Pack.

1. Purpose & Requirement

How To Print is the first tab and the "front door" of the workbook. It holds no readings. Instead, it teaches the routine you will repeat for years: record at home, update the labs, keep the medicine list current, and then send the **Doctor Pack** before each visit.

- **Why it exists:** a new user — or a caregiver who steps in — can run the whole workbook from this single page.
- **What it compiles:** the five-step data routine, the colour codes, a pre-visit checklist, a description of every sheet, and two ways to produce the Doctor Pack (PDF or paper).
- **Key message:** send the doctor only the **three ★ sheets** (about six pages). Sending the whole workbook adds eight or more pages of raw tables, hides the key findings and shares more personal data than needed.



NEPHRO HEALTHCARE TRACKER (PERSONAL) - Ver 2.0

Read all Instructions very carefully before commencing Data Entry in any Datasheet

Record Vitals at Home → Update Lab Reports { prior to each Consultation / Doctor's visit / Before Every Follow-up

YOUR VITALS DATA ROUTINE — 5 SIMPLE STEPS

1. **Once:** Fill Patient Info (yellow cells). Every other sheet reads from it.
2. **Every day:** Record / enter BP, Pulse, SpO_2, Weight, Fluids and Sugar (FBS) in Daily Vitals Log.
3. **At Each Lab test / Visit:** Type the test date in row 4 of Lab Results (oldest first, left to right, no empty column), then the results below it in relevant cells.
4. **When New Medicines are Added / Medicines Change:** Add a new line (For New Meds) in Medication History and set Status = ACTIVE. If dosage of a particular medicine is Incr / Decr, mark it as CHANGED and put the Date of change at relevant place (column J). Add a fresh Medication Entry [With Revised Data] in a Blank New Row and set its Status = ACTIVE. Add a remark suitably in " Clinical Notes / Remarks" Column.
5. **Before each visit:** check Doctor Referral Report, Dashboard and Daily Vitals Log, then send by E-Mail / Carry along : the Doctor Pack (see right).

COLOUR CODES IN USE

- **Yellow cell** = you type here
- **White / grey cell** = works itself out — please don't overwrite
- **Red / orange** = outside the safe limit — INFORM your doctor

BEFORE EVERY CONSULTATION APPOINTMENT / VISIT — CHECKLIST

- ☐ Daily Vitals Log is filled in and is up to today
- ☐ Latest lab results have been entered, dates oldest → newest
- ☐ Medication History is up to date
- ☐ Dashboard Layout and Doctor Referral Report look right
- ☐ Dated copy of this file saved (e.g. Nephro_Tracker_05Oct2026.xlsx)

WHAT EACH SHEET CONTAINS (colour = tab colour, ★ = goes to the doctor)

- How To Print → This guide
- ★ Doctor Referral Report → 1-page summary for the doctor
- ★ Dashboard → Alerts, lab snapshot, 8 trend charts
- Patient Info → Demographics, transplant details, history
- ★ Daily Vitals Log → BP, pulse, SpO_2, weight, fluids, sugar
- Lab Results → All blood & urine tests across dates
- Medication History → Medicine log: start, stop, changes
- Doctor Visit Log → Date of consultation + key values per visit
- Weekly Summary → Weekly averages + alert counts
- Lab Reference Ranges → Post-transplant targets
- Dashboard Settings → Alert limits you can edit

Send by E-Mail prior to your Follow-up / Carry along Prints of the Doctor Pack (Doctor Referral Report + Dashboard + Daily Vitals Log)

DOCTOR PACK (3 SHEETS, ABOUT 6 PAGES) : E-MAIL / BE CARRIED FOR CONSULTATIONS

Send or carry **only these three sheets**. In a 10–15 minute consultation the doctor needs a clear summary first, with the day-by-day readings behind it.

- **Doctor Referral Report (1 page)** — who you are, last-7-day vitals, latest labs, current medicines, space for the doctor's plan.
- **Dashboard (2 pages)** — alerts, creatinine / eGFR / tacrolimus trends, lab snapshot, home BP, weight, urine, SpO_2 and sugar trends.
- **Daily Vitals Log (about 3 pages)** — every day's BP, pulse, SpO_2, weight, fluids, urine and sugar, for the doctor to study in detail.

DOCTOR PACK : OPTION 1 — SAVE AS PDF (RECOMMENDED)

1. Click the **Doctor Referral Report** tab, hold **Ctrl** and click the **Dashboard** and **Daily Vitals Log** tabs — all three are now selected.
2. Press **Ctrl+P** (Windows) or **Cmd+P** (Mac). Printer: Microsoft Print to PDF (Windows) or Save as PDF (Mac).
3. Under Settings choose **Print Active Sheets** — not Print Entire Workbook.
4. Click Print / Save and name the file, e.g. [redacted]
5. Email or WhatsApp the PDF to your doctor before the visit. Then click any other tab to un-select the sheets.

DOCTOR PACK : OPTION 2 — PRINT ON PAPER

▸ Select the same three tabs → **Ctrl+P** → **Print Active Sheets** → choose printer → Print. Carry the pages to the clinic.

ONLY IF THE DOCTOR ASKS — KEEP READY, DON'T SEND ROUTINELY

- **Lab Results** — the full table, to compare older tests
- **Medication History** — to review past dose changes
- **Weekly Summary** — week-by-week averages (1 page)
- **Original hospital lab reports** — the official record (Always better to Carry along during consultation visits)

PLEASE DON'T SEND THE ENTIRE WORKBOOK [By E-mail / Whatsapp]

Sending every sheet adds **8+ more pages** of raw tables (patient info, lab table, medication log, visit log, settings). They bury the key findings, use up the doctor's limited time, and share more personal data than needed. **Keep the full file for your own records.**

Figure 2 The How To Print sheet (patient name removed)

Left half: the five-step routine, colour codes, the pre-visit checklist and the list of sheets with their tab colours (★ = goes to the doctor). Right half: the contents of the Doctor Pack, Option 1 (save as PDF), Option 2 (print), what to keep ready only if the doctor asks, and the red reminder not to send the entire workbook.

2. Visual Layout Blueprint

The sheet is a two-column page. The left column is about **you** (routine, colours, checklist, sheet list). The right column is about **the doctor** (what to send and how).

Map of the How To Print sheet

```
┌──────────────────────────────────────────────────────────────────────────────────────────┐
│  NEPHRO HEALTHCARE TRACKER ( PERSONAL ) - Ver 2.0      [navy banner]                     │
│  Read all Instructions very carefully ...             [violet band]                      │
├──────────────────────────────────────┬───────────────────────────────────────────────────┤
│ ORANGE: Record Vitals at Home -> Update Labs │ ORANGE: Send by E-Mail / Carry along prints │
├──────────────────────────────────────┼───────────────────────────────────────────────────┤
│ YOUR VITALS DATA ROUTINE - 5 SIMPLE STEPS    │ DOCTOR PACK (3 SHEETS, ABOUT 6 PAGES)       │
│ 1 Once : fill Patient Info (yellow cells)    │   * Doctor Referral Report (1 page)         │
│ 2 Every day : Daily Vitals Log               │   * Dashboard (2 pages)                     │
│ 3 Each lab test : Lab Results (row 4 date)   │   * Daily Vitals Log (about 3 pages)        │
│ 4 Medicine added/changed : Medication Hist.  │   [green panel: send only these three]      │
│ 5 Before each visit : check, then send       │ OPTION 1 - SAVE AS PDF (RECOMMENDED)        │
│                                              │   1 Select the 3 tabs (Ctrl + click)        │
│                                              │   2 Ctrl+P -> Print to PDF / Save as PDF    │
├──────────────────────────────────────┼───────────────────────────────────────────────────┤
│ COLOUR CODES IN USE                          │   3 "Print Active Sheets"  4 Name & Save    │
│   [cream] Yellow = you type here             │   5 E-mail / WhatsApp the PDF               │
│   [grey ] White/grey = works itself out      │ OPTION 2 - PRINT ON PAPER                   │
│   [red  ] Red/orange = INFORM your doctor    │   same 3 tabs -> Ctrl+P -> Print            │
├──────────────────────────────────────┼───────────────────────────────────────────────────┤
│ BEFORE EVERY VISIT - CHECKLIST               │ ONLY IF THE DOCTOR ASKS (keep ready)        │
│   [ ] Vitals log up to today                 │   Lab Results, Medication History,          │
│   [ ] Latest labs entered (old -> new)       │   Weekly Summary, original lab reports      │
│   [ ] Medication History up to date          │ [red bar] PLEASE DO NOT SEND THE ENTIRE     │
│   [ ] Dashboard & Referral Report look right │           WORKBOOK  (e-mail / WhatsApp)     │
│   [ ] Dated copy saved                       │   8+ extra pages, more personal data        │
├──────────────────────────────────────────────┴───────────────────────────────────────────┤
│  WHAT EACH SHEET CONTAINS  (colour swatch = tab colour,  * = goes to the doctor)         │
│  How To Print | * Referral Report | * Dashboard | Patient Info | * Daily Vitals Log | Lab Results │
│  Medication History | Doctor Visit Log | Weekly Summary | Lab Reference Ranges | Dashboard Settings │
└──────────────────────────────────────────────────────────────────────────────────────────┘
```

3. Color-Coded Guidance

This sheet introduces the three colour codes used everywhere in the workbook, and uses its own banner colours to group information.

Colour	What you see	What it means / what to do
Aa 123	**Pale yellow cell**	**You type here.** Every sheet uses this colour for entries.
Aa 123	**White / grey cell**	**Works itself out.** Formulas live here — please do not overwrite.
Aa 123	**Red / orange**	**Outside the safe limit — INFORM your doctor.**
Navy	**Navy bars**	Section titles and the main banner.
Orange	**Orange bars**	Instructions you should act on (routine, checklist, “only if the doctor asks”).
Teal	**Teal bars**	Everything about the Doctor Pack.
Violet	**Violet band**	The one-line warning to read the instructions before entering data.

4. Data Input Requirements

There is nothing to type on this sheet. It is read-only guidance. You will, however, use it to carry out these actions:

- **Select the three sheets:** click the **Doctor Referral Report** tab, hold `Ctrl`, then click **Dashboard** and **Daily Vitals Log**.
- **Choose the printer:** *Microsoft Print to PDF* (Windows) or *Save as PDF* (Mac).
- **Choose "Print Active Sheets"** under Settings — never "Print Entire Workbook".
- **Name the file** with the patient name and date, for example `PatientName_Doctor_Pack_05Oct2026.pdf`.

> **UN-SELECT THE SHEETS AFTERWARDS**
> After saving the PDF, click any other tab to un-select the three sheets. If the three sheets stay selected ("grouped"), anything you type next is typed into **all three at once**.

5. Lifecycle & Triggers

When	What to do with this sheet
First day	Read it once, top to bottom, before entering any data.
Before every consultation	Run the checklist and produce the Doctor Pack (Option 1 or 2).
When someone new helps you	Give them this page — it explains the whole routine.
Never	There is no need to edit this sheet.

CHAPTER 2 · INPUT SHEET · COMPLETE ONCE

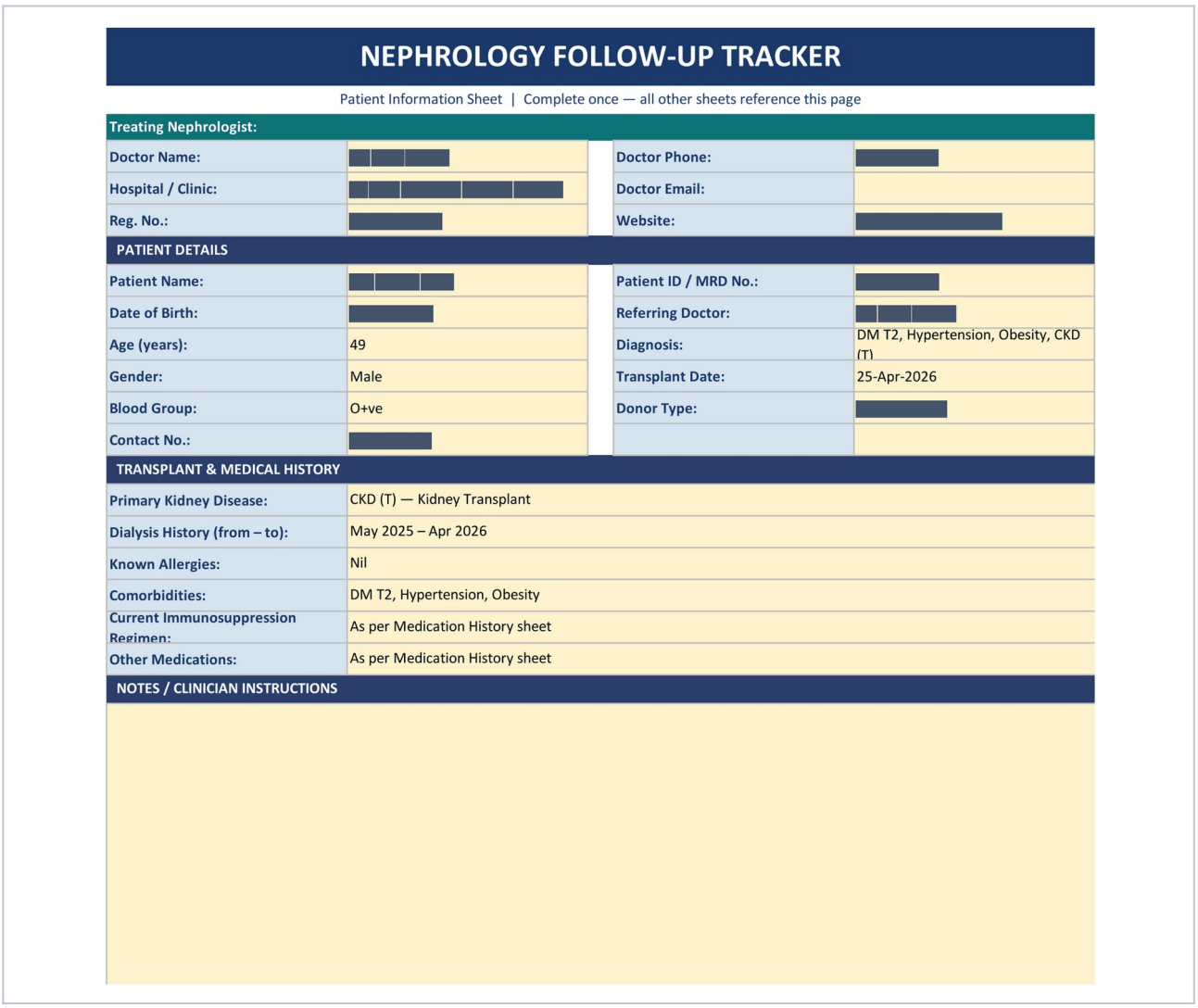
Chapter 2: Patient Info

Complete once — every other sheet reads from this page.

1. Purpose & Requirement

Patient Info is the identity card of the workbook. It records who the patient is, who the treating nephrologist is, and the key facts of the transplant. Other sheets copy this information automatically, so you only ever type it **once**.

- **Feeds the Doctor Referral Report:** patient name, ID, date of birth, gender, blood group, transplant date, diagnosis, comorbidities, and the doctor / hospital line at the top.
- **Feeds the Doctor Visit Log:** the **transplant date** decides which follow-up schedule applies (every 15 days in the first month, monthly until six months, every 60 days until one year, then every 90 days).
- **Holds a notes area** for instructions your clinician wants you to remember. These notes stay on this sheet for your own reference.



NEPHROLOGY FOLLOW-UP TRACKER

Patient Information Sheet | Complete once — all other sheets reference this page

Treating Nephrologist:			
Doctor Name:	[redacted]	Doctor Phone:	[redacted]
Hospital / Clinic:	[redacted]	Doctor Email:	
Reg. No.:	[redacted]	Website:	[redacted]
PATIENT DETAILS			
Patient Name:	[redacted]	Patient ID / MRD No.:	[redacted]
Date of Birth:	[redacted]	Referring Doctor:	[redacted]
Age (years):	49	Diagnosis:	DM T2, Hypertension, Obesity, CKD (T)
Gender:	Male	Transplant Date:	25-Apr-2026
Blood Group:	O+ve	Donor Type:	[redacted]
Contact No.:	[redacted]		
TRANSPLANT & MEDICAL HISTORY			
Primary Kidney Disease:	CKD (T) — Kidney Transplant		
Dialysis History (from – to):	May 2025 – Apr 2026		
Known Allergies:	Nil		
Comorbidities:	DM T2, Hypertension, Obesity		
Current Immunosuppression Regimen:	As per Medication History sheet		
Other Medications:	As per Medication History sheet		
NOTES / CLINICIAN INSTRUCTIONS			

Figure 3 The Patient Info sheet (names, ID, phone numbers, doctor and hospital blanked)

Pale-blue cells are fixed labels; pale-yellow cells are yours to fill. The grey-blue bars hide personal details. Clinical entries such as age, gender, blood group, transplant date and history remain visible so you can see how a completed sheet looks.

2. Visual Layout Blueprint

Map of the Patient Info sheet (cell addresses in square brackets)

```
┌──────────────────────────────────────────────────────────────────────────┐
│  NEPHROLOGY FOLLOW-UP TRACKER                          [navy title bar]  │
│  Patient Information Sheet | Complete once - all other sheets reference this page │
├──────────────────────────────────────────────────────────────────────────┤
│  TREATING NEPHROLOGIST                                 [teal bar]        │
├──────────────────────────────────────┬───────────────────────────────────┤
│ Doctor Name      [C4 ..........]     │ Doctor Phone  [F4 ..........]     │
│ Hospital/Clinic  [C5 ..........]     │ Doctor Email  [F5 ..........]     │
│ Reg. No.         [C6 ..........]     │ Website       [F6 ..........]     │
├──────────────────────────────────────┴───────────────────────────────────┤
│  PATIENT DETAILS                                       [navy bar]        │
├──────────────────────────────────────┬───────────────────────────────────┤
│ Patient Name  [C8 ..........]        │ Patient ID/MRD No. [F8 .......]   │
│ Date of Birth [C9 ..........]        │ Referring Doctor   [F9 .......]   │
│ Age (years)   [C10 .........]        │ Diagnosis          [F10 ......]   │
│ Gender        [C11 .........]        │ Transplant Date    [F11 ......]   │
│ Blood Group   [C12 .........]        │ Donor Type         [F12 ......]   │
│ Contact No.   [C13 .........]        │                                   │
├──────────────────────────────────────┴───────────────────────────────────┤
│  TRANSPLANT & MEDICAL HISTORY                          [navy bar]        │
├──────────────────────────────────────────────────────────────────────────┤
│  Primary Kidney Disease          [C15 ............................]     │
│  Dialysis History (from - to)    [C16 ............................]     │
│  Known Allergies                 [C17 ............................]     │
│  Comorbidities                   [C18 ............................]     │
│  Current Immunosuppression Reg.  [C19  "As per Medication History sheet"] │
│  Other Medications               [C20  "As per Medication History sheet"] │
├──────────────────────────────────────────────────────────────────────────┤
│  NOTES / CLINICIAN INSTRUCTIONS                        [navy bar]        │
│  [ large yellow writing area - free text, kept for your own reference ]  │
└──────────────────────────────────────────────────────────────────────────┘
```

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Aa 123	**Pale-blue label cell**	A fixed heading such as “Patient Name:”. Do not type here.
Aa 123	**Pale-yellow cell**	**You type here.**
Teal	**Teal bar**	“Treating Nephrologist” section heading.
Navy	**Navy bar**	“Patient Details”, “Transplant & Medical History”, “Notes” headings.

4. Data Input Requirements

Fill every pale-yellow cell. Leave a cell empty only if the information truly does not exist.

Treating nephrologist

- **Doctor Name** (`C4`) — text, for example *Dr. A. Kumar*.
- **Doctor Phone** (`F4`) — digits, with the area code if needed.
- **Hospital / Clinic** (`C5`) — text, the name and city.
- **Doctor Email** (`F5`) — text; optional but useful for sending the Doctor Pack.
- **Reg. No.** (`C6`) — the doctor’s medical registration number.
- **Website** (`F6`) — optional.

Patient details

- **Patient Name** (`C8`) — text, as on hospital records.
- **Patient ID / MRD No.** (`F8`) — the hospital file number.
- **Date of Birth** (`C9`) — a date, typed as `16-Aug-1976`.

- **Referring Doctor** (`F9`) — text.
- **Age (years)** (`C10`) — a whole number such as `49`. *It is typed, not calculated: update it on each birthday.*
- **Diagnosis** (`F10`) — short text, for example *DM T2, Hypertension, CKD (T)*.
- **Gender** (`C11`) — text, *Male* or *Female*.
- **Transplant Date** (`F11`) — a date such as `25-Apr-2026`. **Very important** — the visit schedule in the Doctor Visit Log depends on it.
- **Blood Group** (`C12`) — text such as *O+ve*.
- **Donor Type** (`F12`) — text such as *Living related* or *Deceased*.
- **Contact No.** (`C13`) — digits.

Transplant & medical history

- **Primary Kidney Disease** (`C15`), **Dialysis History from – to** (`C16`), **Known Allergies** (`C17`, write *Nil* if none), **Comorbidities** (`C18`) — short text.
- **Current Immunosuppression Regimen** (`C19`) and **Other Medications** (`C20`) — pre-filled with *"As per Medication History sheet"*. Leave as is; the real list lives in Chapter 5.
- **Notes / Clinician Instructions** — large area for free text.

> ★ **TYPE DATES AS DD-MMM-YYYY**
> Typing `16-Aug-1976` is safe on every computer. Typing only numbers such as `08-09-1976` can be read as 8 September or 9 August, depending on your computer's regional settings.

5. Lifecycle & Triggers

Trigger	Action
Day 1 (new workbook)	Fill the whole sheet.
Birthday	Update **Age (years)**.
Change of doctor, hospital or phone number	Update the "Treating Nephrologist" block.
New diagnosis, allergy or comorbidity	Update the history block and tell your doctor.
Start of a new quarterly workbook	Copy the sheet across, or keep it when you reuse the file (see the Blank Personal workbook).

CHAPTER 3 · INPUT SHEET · EVERY DAY ★ DOCTOR PACK

Chapter 3: Daily Vitals Log

One row per day: the heartbeat of your home monitoring.

1. Purpose & Requirement

The **Daily Vitals Log** is the sheet you will use most. Each day you record the same set of measurements, at the **same time of day**. Over weeks, the numbers reveal trends that a single clinic reading can never show: creeping blood pressure, weight gain from fluid, falling oxygen level, or high morning sugar.

- **Metrics compiled:** blood pressure (systolic and diastolic), pulse, SpO_2, temperature, weight, fluid intake, urine output and frequency, swelling (oedema), fasting blood sugar, medicines taken, and free-text symptoms.
- **Automatic checks:** two flag columns (**BP Alert** and **SpO_2 Alert**) tell you when a reading is outside the limits in Dashboard Settings.
- **Feeds:** the Dashboard (alerts, four vitals charts, sugar block), the Weekly Summary, the 7-day table of the Referral Report, and the vitals readout of the Doctor Visit Log.
- **Goes to the doctor** ★ as part of the Doctor Pack.

DAILY HOME MONITORING LOG — KIDNEY TRANSPLANT PATIENT

Fill every day at the SAME time. Yellow cells = mandatory. Alert doctor immediately for RED values. Bring this log to every visit.

Date	BP Systolic (mmHg)	BP Diastolic (mmHg)	Pulse (bpm)	SpO_2 (%)	Temp (°F)	Weight (kg)	Fluid In (mL)	Urine Out (mL)	Urine Freq (#/day)	Oedema? (Y/N)	Blood Sugar (mg/dL)	Meds Taken?	Symptoms / Notes	BP Alert	SpO_2 Alert
30-Jul-2026	133	70	92	98	NT	90.05	3500	3130	8	N	145	Y	Old Presc	OK	OK
31-Jul-2026	138	82	82	98	NT	89.60	3000	2930	7	N		Y	Old Presc	OK	OK
01-Aug-2026	125	76	89	98	NT	90.00	3500	3140	8	N		Y	Old Presc	OK	OK
02-Aug-2026	139	83	85	98	NT	90.10	4000	3660	3	N		Y	Old Presc	OK	OK
03-Aug-2026	134	80	93	97	NT	90.00	4000	3610	9	N		Y	Old Presc	OK	OK
04-Aug-2026	136	80	85	98	NT	90.35	3500	3440	8	N	150	Y	Old Presc	OK	OK
05-Aug-2026	135	81	84	97	NT	90.05	3000	2980	7	N	107	Y	Old Presc	OK	OK
06-Aug-2026	132	77	86	99	NT	90.05	3000	3060	8	N	117	Y	Old Presc	OK	OK
07-Aug-2026	117	85	81	96	NT	90.00	3000	3190	8	N	116	Y	Old Presc	OK	OK
08-Aug-2026	123	75	83	97	NT	90.05	3500	3400	8	N	130	Y	Rev Presc	OK	OK
09-Aug-2026	117	74	84	97	NT	90.00	3500	3450	9	N	169	Y	Rev Presc	OK	OK
10-Aug-2026	137	76	84	98	NT	89.35	3300	3240	8	N	170	Y	Rev Presc	OK	OK
11-Aug-2026	131	81	79	99	NT	88.80	3000	2800	7	N	101	Y	Rev Presc	OK	OK
12-Aug-2026	121	77	85	97	NT	89.40	3200	3190	9	N	98	Y	Rev Presc	OK	OK
13-Aug-2026	133	80	89	97	NT	89.45	2800	2660	7	N	153	Y	Rev Presc	OK	OK
14-Aug-2026	132	85	87	99	NT	89.30	3000	2920	8	N	129	Y	Rev Presc	OK	OK
15-Aug-2026	123	79	77	99	NT	89.30	3200	3180	8	N	117	Y	Rev Presc	OK	OK
16-Aug-2026	136	79	82	98	NT	89.30	3000	2900	7	N	143	Y	Rev Presc	OK	OK
17-Aug-2026	131	86	78	98	NT	89.40	3300	3290	7	N	102	Y	Rev Presc	OK	OK
18-Aug-2026	119	69	80	99	NT	89.25	3300	3230	9	N	84	Y	Rev Presc	OK	OK
19-Aug-2026	133	83	79	98	NT	89.15	2800	2620	8	N	73	Y	Rev Presc	OK	OK
20-Aug-2026	125	86	80	98	NT	89.05	3500	3570	7	N	112	Y	Rev Presc	OK	OK
21-Aug-2026	116	69	70	96	NT	89.30	3500	3350	8	N	137	Y	Rev Presc	OK	OK

Figure 4 The Daily Vitals Log with readings

The first rows of a real log. Pale-yellow cells are the mandatory entries. The right-hand columns (BP Alert, SpO_2 Alert) show OK or an alert. The Fluid In column carries a green-to-red colour scale. Blood sugar is left blank on days it was not measured.

2. Visual Layout Blueprint

Map of the Daily Vitals Log (Sys = systolic, Dia = diastolic, FlIn = Fluid In, UrOut = Urine Out)

```
DAILY HOME MONITORING LOG - KIDNEY TRANSPLANT PATIENT                    [row 1: navy title bar]
Fill every day at the SAME time. Yellow cells = mandatory. Alert doctor for RED values.  [row 2]

   B     C     D     E     F     G     H     I     J     K     L     M     N     O     P     Q   [columns]
Date   Sys   Dia   Pulse SpO2  Temp  Wt    FlIn  UrOut Freq  Oed?  Sugar Meds  Notes BPAl  SpAl  [row 3: headers]

30-Jul 133   70    92    98    NT    90.05 3500  3130  8     N     145   Y     ....  OK    OK    [row 4 = day 1]
31-Jul 138   82    82    98    NT    89.60 3000  2930  7     N     --    Y     ....  OK    OK    [row 5 = day 2]
  :     :     :     :     :     :     :     :     :     :     :     :     :     :     :     :
  ONE ROW PER DAY  -  110 rows (rows 4 to 113)  -  yellow = you type;  P and Q = automatic flags

COLOUR LEGEND (printed under the table): Yellow = mandatory | Red BP | Red SpO2 | Orange Sugar | Notes
```

Date	BP Systolic (mmHg)	BP Diastolic (mmHg)	Pulse (bpm)	SpO_2 (%)	Temp (°F)	Weight (kg)	Fluid In (mL)	Urine Out (mL)	Urine Freq (#/day)	Oedema? (Y/N)	Blood Sugar (mg/dL)	Meds Taken?	Symptoms / Notes	BP Alert	SpO_2 Alert

*COLOUR LEGEND: Yellow = Mandatory entry | Red BP = outside the BP limits in Dashboard Settings (default: Systolic >140 or <90, Diastolic >90) | Red SpO_2 = <94% (call doctor) | Orange Sugar = above alert limit in Dashboard Settings (default >180 mg/dL) | * Blood Sugar = fasting value before breakfast | Fluid In = all liquids consumed | Urine Out = total 24-hr collection*

Figure 5 The empty end of the log and its colour legend

Empty rows show how a new row will look: Date, BP Diastolic, Pulse, Weight and Fluid In in pale yellow; BP Systolic and SpO_2 appear pink while empty. The colour legend is printed beneath the table.

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Aa 123	**Pale-yellow cell**	Mandatory entry: Date, BP Systolic, BP Diastolic, Pulse, SpO_2, Weight, Fluid In.
Aa 123	**Pink cell**	A BP Systolic or SpO_2 cell that is **empty** looks pink (the sheet treats "nothing" as "too low"). It becomes normal once a valid number is typed. A **typed** value shown in red means it is outside the limit.
Aa 123	**Red BP or red SpO_2**	BP above the high limit or below the low limit; SpO_2 below 94%. **Inform your doctor.**
Aa 123	**Orange / amber sugar**	Blood sugar above the alert limit (default 180 mg/dL).
Aa 123	**Green → yellow → red (Fluid In)**	A relative "heat map" of intake across the log: lowest amounts green, highest red. It compares days with each other; it is **not** a medical alert.
Aa 123	**White cell**	Optional entries (Temp, Urine, Notes…) and the two automatic flag columns.
Navy	**Navy header**	Column titles and units.

4. Data Input Requirements

Type **one row per day**, in date order, with no empty rows between days. Start at row 4.

- **Date** (`B`) — a date, `dd-mmm-yyyy`. Mandatory. Rows must follow each other day by day.
- **BP Systolic** (`C`) and **BP Diastolic** (`D`) — whole numbers in mmHg, for example `128` and `81`. Mandatory. Measure after resting for five minutes.
- **Pulse** (`E`) — whole number, beats per minute. Mandatory.
- **SpO_2** (`F`) — whole number, %, for example `98`. Mandatory.
- **Temp** (`G`) — a number in °F such as `98.6`, or **NT** if not taken. Optional.
- **Weight** (`H`) — kilograms with up to two decimals, for example `89.45`. Mandatory. Use the same scale, same clothing, same time (ideally after the toilet, before breakfast).
- **Fluid In** (`I`) — total of **all** liquids in the last 24 hours, in mL. Mandatory.
- **Urine Out** (`J`) — total 24-hour urine collection, in mL.
- **Urine Freq** (`K`) — how many times you passed urine in 24 hours (whole number).
- **Oedema?** (`L`) — type `Y` or `N` for swelling of feet, ankles or face.
- **Blood Sugar** (`M`) — mg/dL, **fasting, before breakfast**. Leave blank if not measured.
- **Meds Taken?** (`N`) — `Y` if every dose was taken, otherwise `N` and explain in Notes.
- **Symptoms / Notes** (`O`) — short text: “mild ankle swelling”, “missed evening dose”, “new prescription”.
- **BP Alert** (`P`) and **SpO_2 Alert** (`Q`) — automatic; do not type.

> **EMPTY IS BETTER THAN GUESSED**
> If you did not measure something, leave the cell **empty** (or write NT in Temp). Never type 0. A zero would be counted as a real reading and would spoil the averages and charts.

> **CAPACITY OF THE LOG**
> The log has **110 rows** (rows 4 to 113 — about three and a half months). The Dashboard’s four vitals charts plot the **first 95 rows**; the alert tiles, the weekly averages and the 7-day table use every row. Start a fresh workbook for each quarter.

5. Lifecycle & Triggers

When	What to do
Every day, same time	Add one new row with the day’s readings.
When an alert flag appears	Re-measure after resting; if it is confirmed, contact your doctor.
Once a week	Look at the Weekly Summary (Chapter 8) to see the trend.
Before each visit	Check the log is “up to today”, then send/print it as part of the Doctor Pack.
Log full (row 113) or new quarter	Save the file as an archive and begin a new workbook.

CHAPTER 4 · INPUT SHEET · AFTER EACH LAB TEST

Chapter 4: Lab Results

All your blood and urine tests, side by side across dates.

1. Purpose & Requirement

Lab Results is a serial table of every laboratory test, one column per test date. Reading across a row shows how one value (for example creatinine) changed over time. Reading down a column shows everything that was measured on one day.

- **Metrics compiled:** 28 tests in seven groups — renal function, electrolytes, metabolic/glucose, blood count, liver and metabolic, immunosuppression/viral monitoring, and urine tests.
- **Capacity:** up to **20 test dates** (columns G to Z).
- **Feeds:** the Dashboard (three lab charts, lab snapshot, alert tiles), the Referral Report (latest results) and the lab part of the Doctor Visit Log readout.
- **Reference columns** beside each test show the unit, the general normal range, the post-transplant target and the usual testing frequency.

BLOOD & URINE TEST LOG — SE

HOW TO ENTER: type each test date in row 4 (DATE →), moving LEFT → RIGHT and OLDEST FIRST, with no empty column in between (out-of-order dates are bloc

Test / Parameter	Unit	Normal Range (General)	TX Target (Post-Tx)	Frequency	Test Date 1 (enter date →)	Test Date 2 (enter date →)	Test Date 3 (enter date →)	Test Date 4 (enter date →)	Test Date 5 (enter date →)	Test Date 6 (enter date →)
DATE →					9-Jul-26	30-Jul-26	28-Aug-26	29-Sep-26	1-Oct-26	
RENAL FUNCTION										
Serum Creatinine	mg/dL	0.6 – 1.2	Stable baseline	Monthly	1.48	1.65	1.68		1.57	
eGFR	mL/min	≥ 60	≥ 45	Monthly	57		49		53	
Tacrolimus Trough	ng/mL	—	Early: 8–12 | Late: 5–8	Monthly	5.53	12	11.8	10.7		
ELECTROLYTES										
Calcium (Total)	mg/dL	8.5 – 10.5	8.5 – 10.2	Monthly		9	9.5		8.1	
Phosphorus	mg/dL	2.5 – 4.5	2.5 – 4.5	Monthly		2.4	3		3	
Uric Acid	mg/dL	2.4 – 7.0	< 7.0	Monthly		7.5	5.5		5.8	
Potassium	mEq/L	3.5 – 5.0	3.5 – 5.0	Monthly		4.1	3.9		4.3	
Sodium	mEq/L	135 – 145	135 – 145	Monthly		134	139		141	
Serum Bicarbonate	mEq/L	22 – 29	22 – 29	Monthly						
METABOLIC / GLUCOSE										
FBS (Fasting Sugar)	mg/dL	70 – 100	70 – 126	Monthly		145.9				
PPBS (Post-meal)	mg/dL	≤ 140	≤ 180	Monthly		205.44				
HbA1c	%	≤ 5.7	≤ 7.0	3-Monthly			7.5			
COMPLETE BLOOD COUNT										
Haemoglobin	g/dL	12 – 17	11 – 14	Monthly		11.5	12.8		13.6	
TLC (WBC)	$\times 10^3/\mu L$	4 – 11	4 – 10	Monthly		10.9	9.9		7.2	
Platelets	$\times 10^3/\mu L$	150 – 400	150 – 350	Monthly		325	318		279	
LIVER & METABOLIC										
Serum Albumin	g/dL	3.5 – 5.0	3.5 – 5.0	Monthly		3.5	3.7		3.8	
Total Bilirubin	mg/dL	≤ 1.2	≤ 1.2	3-Monthly					0.5	
SGOT / AST	U/L	≤ 40	≤ 40	3-Monthly					41	
SGPT / ALT	U/L	≤ 56	≤ 56	3-Monthly					65	
Lipid Profile (LDL)	mg/dL	≤ 100	≤ 100	3-Monthly			92.2		81	
IMMUNOSUPPRESSION MONITORING										
Vitamin D (25-OH)	ng/mL	30 – 100	30 – 100	3-Monthly						
BKV PCR	copies/mL	Negative	< 10,000	Monthly × 9m then 3m		Below Detection Limit				
CMV PCR	copies/mL	Negative	Undetectable	Monthly × 6m						
URINE TESTS										
Urine RE (Protein)	-	—	Nil / Trace	Monthly		Absent (Nil)	Not Detected (Nil)			
Urine RE (RBC/WBC)	cells/hpf	≤ 5	≤ 5	Monthly		0 RBC / 4 WBC	0 RBC / 0 WBC			
Urine C/S	-	—	No Growth	If symptomatic						
Urine PCR (UACR)	mg/g	< 150	< 300	Monthly		35.6	24.1		33.3	
24h Urine Protein	mg/24h	< 150	< 300	If indicated						
DOCTOR'S NOTES ON LATEST RESULTS										

Figure 6 The Lab Results sheet (first six of twenty test-date columns)

Each test date (row 4, yellow) heads a column. Results are typed beneath. Blank cells mean "not tested that day". Words are allowed for non-numeric tests, for example "Below Detection Limit" or "Not Detected (Nil)".

2. Visual Layout Blueprint

Map of the Lab Results sheet

```
BLOOD & URINE TEST LOG - SERIAL LAB RESULTS                          [navy title bar]
HOW TO ENTER: type each test date in row 4, LEFT -> RIGHT, OLDEST FIRST, no empty column in between

   B                C      D           E            F          G         H         ...      Z
Test / Parameter    Unit   Normal      TX Target    Frequency  Test      Test                Test
                           (General)   (Post-Tx)               Date 1    Date 2    ...      Date 20

DATE ->                                                      [9-Jul-26][30-Jul-26] ... [yellow date cells]

RENAL FUNCTION                                               [group bar]
Serum Creatinine    mg/dL  0.6 - 1.2   Stable base. Monthly    1.48      1.65      ...  [results]
eGFR                mL/min >= 60       >= 45        Monthly    57                  ...
Tacrolimus Trough   ng/mL  --          Early 8-12   Monthly    5.53      12        ...
ELECTROLYTES | METABOLIC / GLUCOSE | COMPLETE BLOOD COUNT | LIVER & METABOLIC | IMMUNOSUPPRESSION | URINE
   ...  28 tests in all  (rows 6 to 39)  ...

DOCTOR'S NOTES ON LATEST RESULTS   [free-text notes area below the table]
```

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Aa 123	**Pale-yellow date cell (row 4)**	**You type each test date here.** The sheet blocks out-of-order dates.
Aa 123	**Pale-blue label cells**	Test name, unit and reference ranges. Fixed text.
Navy	**Navy group bars**	RENAL FUNCTION, ELECTROLYTES, METABOLIC / GLUCOSE, and so on.
Teal	**Teal "Test Date n" headings**	Column titles for the 20 test dates.
Aa 123	**Light-green result cells**	In the example, cells that hold a result are shaded and bold so that completed tests stand out. It is a "filled" marker, **not** a medical judgement.
Aa 123	**White cell**	An empty result — nothing was tested.

WHERE IS "NORMAL / HIGH / LOW" DECIDED?

This sheet stores the numbers. The automatic judgement (▲HIGH, ▼LOW, ✔ In range) appears in the **Lab Snapshot** on the Dashboard (Chapter 6), using the limits in Dashboard Settings (Chapter 11).

4. Data Input Requirements

- **Test date** (row 4, from `G4` rightwards) — a date, `dd-mmm-yyyy`. Enter dates **left to right, oldest first**, with **no empty column** in between. An out-of-order or skipped date is refused with a message.
- **Result cells** (rows 6 to 39) — numbers (for example `1.57`) in the unit printed in column C, or short text for qualitative tests (`Negative`, `Not Detected (Nil)`, `0 RBC / 4 WBC`).
- **Units matter:** for example platelets are in $\times 10^3/\mu L$ — a report that says *2.79 lakh* should be entered as `279`; a white-cell count of *7200/cumm* is entered as `7.2`.

- **Not tested:** leave the cell empty.
- **Doctor's Notes on Latest Results** — optional free text, for example flags raised by the laboratory, or tests that have no row in the table.

NEVER LEAVE A GAP BETWEEN DATES

If you skip a column, later dates will not be accepted and the charts will break. If a test date is missing, leave its **results** blank but still type the date.

COMBINE REPORTS FROM THE SAME PERIOD

If several reports were collected within a few days, enter them under **one** test date and describe this in the notes area. It keeps the trend lines clean.

5. Lifecycle & Triggers

When	What to do
Each time you receive lab reports	Add a new date in the next free column and enter the results.
Before each visit	Confirm the newest report has been entered (Checklist, Chapter 1).
Monthly (most post-transplant tests)	Expect one new column per month; 3-monthly tests appear every third month.
All 20 columns used	Start a new workbook and copy the last two or three columns across for continuity.

CHAPTER 5 · INPUT SHEET · WHEN A MEDICINE CHANGES

Chapter 5: Medication History

A single log of every medicine you take, stop or change.

1. Purpose & Requirement

Medicines are the backbone of life after a transplant, and doses change often. **Medication History** keeps one **unified log**: what you take now, what was stopped, what was changed, when, and why. The doctor sees only the **current (ACTIVE)** list on the Referral Report, while the full history stays here.

- **Metrics compiled:** brand name, generic composition, category, dose, timing, route, start date, status, stop date and remarks.
- **Capacity:** 200 rows (rows 5 to 204).
- **Feeds:** the "Current Medications" table on the Doctor Referral Report (up to 15 ACTIVE medicines, with a count).
- **Never delete** a row for a stopped medicine. Change its status instead — the history is valuable.

MEDICATION HISTORY — UNIFIED LOG (Current Medications + All Changes)

Add a new row for every medication added, changed, or stopped. Status: ACTIVE | CHANGED | STOPPED

Last Prescription Date: 27-Jun-2026 Prescribing Doctor:

Brand Name	Composition / Generic	Category	Dose	Timing & Frequency	Route	Start Date	Status	Stop Date	Clinical Notes / Remarks
Mycept S 360mg (Mycophenolate Na)	Mycophenolate Sodium 360 mg	Immunosuppressant (MPA)	360 mg	1-0-1 (After Food)	Oral	05-Jun-2026	ACTIVE		Anti-rejection; monitor TLC — leukopenia risk
Pangraf 4.0 mg (Tacrolimus)	Tacrolimus 4 mg	Immunosuppressant (CNI)	4 mg	1-0-1 (After Food)	Oral	05-Jun-2026	STOPPED	27-Jun-2026	Dose guided by trough level (Tac C0); check renal function
Wysolone 5mg (Prednisolone)	Prednisolone 5 mg	Corticosteroid	5 mg	1-0-0 (After Food)	Oral	05-Jun-2026	ACTIVE		Morning dose; monitor blood sugar — PTDM risk
Vagacyte 450mg (Valganciclovir)	Valganciclovir 450 mg	Antiviral Prophylaxis (CMV)	450 mg	0-0-1 (After Food)	Oral	05-Jun-2026	ACTIVE		CMV prophylaxis; monitor CBC
Candidnorm Drops (Antifungal)	Antifungal oral drops	Antifungal Prophylaxis	4 drops	0-0-0-4 (After Food)	Oral	05-Jun-2026	STOPPED	31-Jul-2026	Oral candidiasis prophylaxis
Septran DS 800/160mg (Co-trimoxazole)	Sulfamethoxazole 800mg + Trimethoprim 160mg	Antibiotic Prophylaxis (PCP)	800/160 mg	0-0-0.5 (After Dinner)	Oral	05-Jun-2026	STOPPED	31-Jul-2026	PCP prophylaxis; alternate day × 10 days; alternate: Sepamax DS
Ondero Met 2.5/500mg (Linagliptin+Metformin)	Linagliptin 2.5mg + Metformin 500mg	Antidiabetic (DPP-4 + Biguanide)	2.5/500 mg	1-0-1 (Before Food)	Oral	05-Jun-2026	STOPPED	27-Jun-2026	PTDM management; caution if eGFR <45
Lupinsulin 30:70 Injection	Human Insulin 30% + NPH Human Insulin 70%	Antidiabetic (Insulin)	38U-0-22U	Before Food - Daily	Subcutaneous	05-Jun-2026	ACTIVE		20-30 min before meals. Store in fridge 2-8°C
Concor AM 2.5mg (Bisoprolol+Amlodipine)	Amlodipine 5mg + Bisoprolol 2.5mg	Antihypertensive (CCB + BB)	2.5/5 mg	1-0-1 (After Food)	Oral	05-Jun-2026	STOPPED	27-Jun-2026	BP target <130/80 mmHg; hold if pulse <50 bpm
Olympviq 4mg Pre-filled Pen	4 mg/dose	Immunomodulator / Biologic	4 mg	Weekly	Subcutaneous	05-Jun-2026	STOPPED	27-Jun-2026	0.25 mg/week SUBCUT; store in fridge; trained administrator
Pangraf 2.5 mg (Tacrolimus)	Tacrolimus 2+0.5 mg	Immunosuppressant (CNI)	2.5 mg	1-0-1 (After Food)	Oral	27-Jun-2026	CHANGED	31-Jul-2026	Dose DECREASED;
Ondero Met 2.5/500mg (Linagliptin+Metformin)	Linagliptin 2.5mg + Metformin 500mg	Antidiabetic (DPP-4 + Biguanide)	2.5/500 mg	1-0-1 (Before Food)	Oral	27-Jun-2026	STOPPED	25-Jul-2026	PTDM management; caution if eGFR <45
Olympviq 4mg Pre-filled Pen	4 mg/dose	Immunomodulator / Biologic	4 mg	Weekly	Subcutaneous	27-Jun-2026	STOPPED	31-Jul-2026	0.5 mg/week SUBCUT; store in fridge; trained administrator
Eurepa V 1/0.3mg Tablet	Repaglinide (1 mg) + Voglibose (0.3 mg)	Antidiabetic	1 Tablet	Twice a day (After Lunch & Dinner)	Oral	27-Jun-2026	ACTIVE		Take immediately before or up to 15-30 min before Meal. Prevents sharp BS spikes after meals.
Dilzem CD 90 mg Capsule	Diltiazem Hydrochloride (90 mg)	Antihypertensive / Calcium Channel Blocker	1 Tablet	Once a day (Morning)	Oral	27-Jun-2026	ACTIVE		After breakfast. Do not crush or chew the ER capsule.
Nebicard 2.5mg	Nebivolol (2.5 mg)	Antihypertensive / Beta-Blocker	1 Capsule	Once a day (Night)	Oral	27-Jun-2026	ACTIVE		After dinner. Helps manage BP and heart rate.
GLUCRETA M 10 / 500MG TABLET	Dapagliflozin 10 MG + Metformin 500 MG	An Anti-diabetic combination	1 Tablet	Once a day (Morning)	Oral	31-Jul-2026	ACTIVE		After breakfast. Do not crush or chew the Tablet.
Pangraf 2.25 mg (Tacrolimus)	Tacrolimus 2+0.25 mg	Immunosuppressant (CNI)	2.25 mg	1-0-1 (After Food)	Oral	27-Jun-2026	ACTIVE	31-Jul-2026	Dose guided by trough level (Tac C0); check renal function [Dose Decr]

Figure 7 The Medication History log (prescribing doctor blanked)

Each row is one medicine at one dose. The Status column is a dropdown coloured green (ACTIVE), amber (CHANGED) or red (STOPPED). When a dose changes, the old row is marked CHANGED and a new ACTIVE row is added with the revised dose.

2. Visual Layout Blueprint

Map of the Medication History sheet

```
MEDICATION HISTORY - UNIFIED LOG (Current Medications + All Changes)          [navy title bar]
Add a new row for every medication added, changed, or stopped. Status: ACTIVE | CHANGED | STOPPED
Last Prescription Date: [E3 .........]                  Prescribing Doctor: [J3 .........]
```

B	C	D	E	F	G	H	I	J	K
Brand Name	Composition / Generic	Category	Dose	Timing & Frequency	Route	Start Date	Status (list)	Stop Date	Clinical Notes / Remarks
Mycept S..	Mycophenolate	Immunosup.	360 mg	1-0-1	Oral	05-Jun	ACTIVE		Anti-rejection ...
Pangraf 4	Tacrolimus 4	Immunosup.	4 mg	1-0-1	Oral	05-Jun	STOPPED	27-Jun	Dose guided by ...
Pangraf 2.5	Tacrolimus ..	Immunosup.	2.5mg	1-0-1	Oral	27-Jun	CHANGED	31-Jul	Dose DECREASED
: one row per medicine / per change - up to 200 rows (rows 5 to 204)									

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Aa 123	**Green status — ACTIVE**	You are taking this medicine now. It appears on the Referral Report.
Aa 123	**Amber status — CHANGED**	An earlier dose that has been replaced. Keep it for history.
Aa 123	**Red status — STOPPED**	The medicine was stopped. Add the stop date.
Aa 123	**White cell**	Free typing (brand, dose, remarks).
Navy	**Navy header**	Column titles.

4. Data Input Requirements

- **Last Prescription Date** (`E3`) — date of the latest prescription, `dd-mmm-yyyy`.
- **Prescribing Doctor** (`J3`) — text.
- **Brand Name** (`B`) — text, as on the prescription.
- **Composition / Generic** (`C`) — the active ingredient and strength.
- **Category** (`D`) — for example *Immunosuppressant (CNI)*, *Antihypertensive*, *Antidiabetic*.
- **Dose** (`E`) — text such as `360 mg` or `1 Tablet`.
- **Timing & Frequency** (`F`) — for example `1-0-1 (After Food)`: morning–noon–night.
- **Route** (`G`) — *Oral*, *Subcutaneous* and so on.
- **Start Date** (`H`) — date.
- **Status** (`I`) — choose from the dropdown: **ACTIVE**, **CHANGED** or **STOPPED**.
- **Stop Date** (`J`) — date the medicine was stopped, or the date its dose was changed.
- **Clinical Notes / Remarks** (`K`) — why it was started or changed.

How to record a dose change (step by step)

DOSE INCREASED OR DECREASED

1. Find the row of the old dose. Set its **Status = CHANGED** and type the **date of change** in the Stop Date column (`J`).
2. In the next blank row, add the medicine again with the **new dose**. Set **Status = ACTIVE** and the new start date.
3. In Remarks write what changed, for example “Dose decreased” or “Revised to 0.5 mg/week”.

REFERRAL REPORT HOLDS 15 MEDICINES

The Doctor Referral Report lists up to **15** ACTIVE medicines. If you have more, its heading shows a warning ("… more not shown — see Medication History"). Nothing is lost; the full list stays on this sheet.

5. Lifecycle & Triggers

Trigger	Action
New medicine prescribed	Add a new row, Status = ACTIVE.
Dose changed	Mark the old row CHANGED (with date); add a new ACTIVE row.
Medicine stopped	Set Status = STOPPED and fill the Stop Date.
After every doctor's visit	Update **Last Prescription Date** and check every row against the new prescription.
Before each visit	Confirm that all ACTIVE rows match what you actually take.

CHAPTER 6 · AUTOMATIC SHEET · 2 PAGES ★ DOCTOR PACK

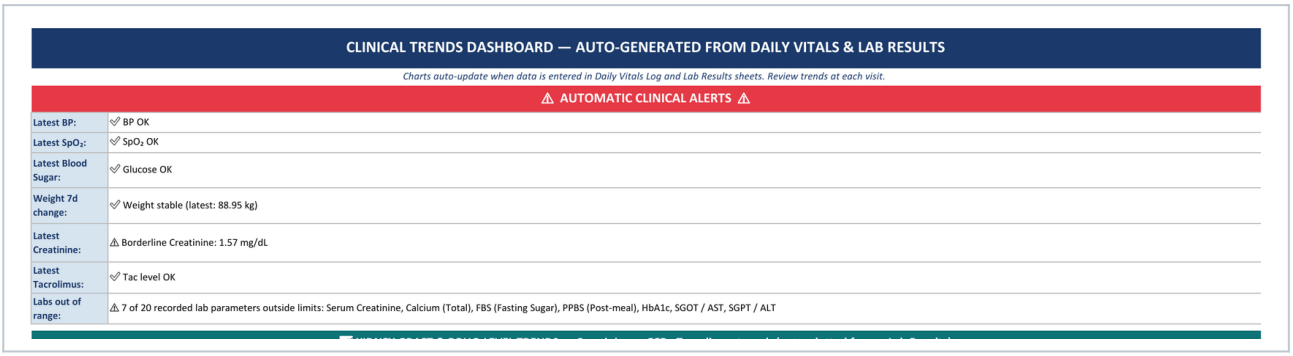
Chapter 6: Dashboard

Alerts, trends and a lab snapshot — your health at a glance.

1. Purpose & Requirement

The **Dashboard** turns rows of numbers into pictures and plain-English messages. Nothing is typed here. Every tile, table and chart is rebuilt automatically whenever you add readings to the **Daily Vitals Log** or results to **Lab Results**.

- **Seven alert tiles** summarise the most important facts: latest blood pressure, SpO_2, blood sugar, 7-day weight change, creatinine, tacrolimus, and how many lab parameters are outside their limits.
- **Three kidney-graft charts** show creatinine, eGFR and tacrolimus trough over your test dates, with the limit lines drawn in.
- **A lab snapshot** compares the latest result with the previous one for 23 tests and states whether each is high, low or in range.
- **Four daily-vitals charts** (weight, urine output, blood pressure, SpO_2) and a **fasting blood-sugar block** with 7-day and 30-day summaries.
- **Two printed pages** — the second of the three ★ Doctor Pack sheets.



CLINICAL TRENDS DASHBOARD — AUTO-GENERATED FROM DAILY VITALS & LAB RESULTS

Charts auto-update when data is entered in Daily Vitals Log and Lab Results sheets. Review trends at each visit.

⚠ AUTOMATIC CLINICAL ALERTS ⚠

Latest BP:	✓ BP OK
Latest SpO_2:	✓ SpO_2 OK
Latest Blood Sugar:	✓ Glucose OK
Weight 7d change:	✓ Weight stable (latest: 88.95 kg)
Latest Creatinine:	⚠ Borderline Creatinine: 1.57 mg/dL
Latest Tacrolimus:	✓ Tac level OK
Labs out of range:	⚠ 7 of 20 recorded lab parameters outside limits: Serum Creatinine, Calcium (Total), FBS (Fasting Sugar), PPBS (Post-meal), HbA1c, SGOT / AST, SGPT / ALT

Figure 8 Automatic Clinical Alerts (the first block of page 1)

Each row is one tile in plain language, marked ✅ when all is well or ⚠️ when attention is needed. Here the creatinine tile warns of a "borderline" value of 1.57 mg/dL, and seven laboratory parameters are listed as outside their limits.

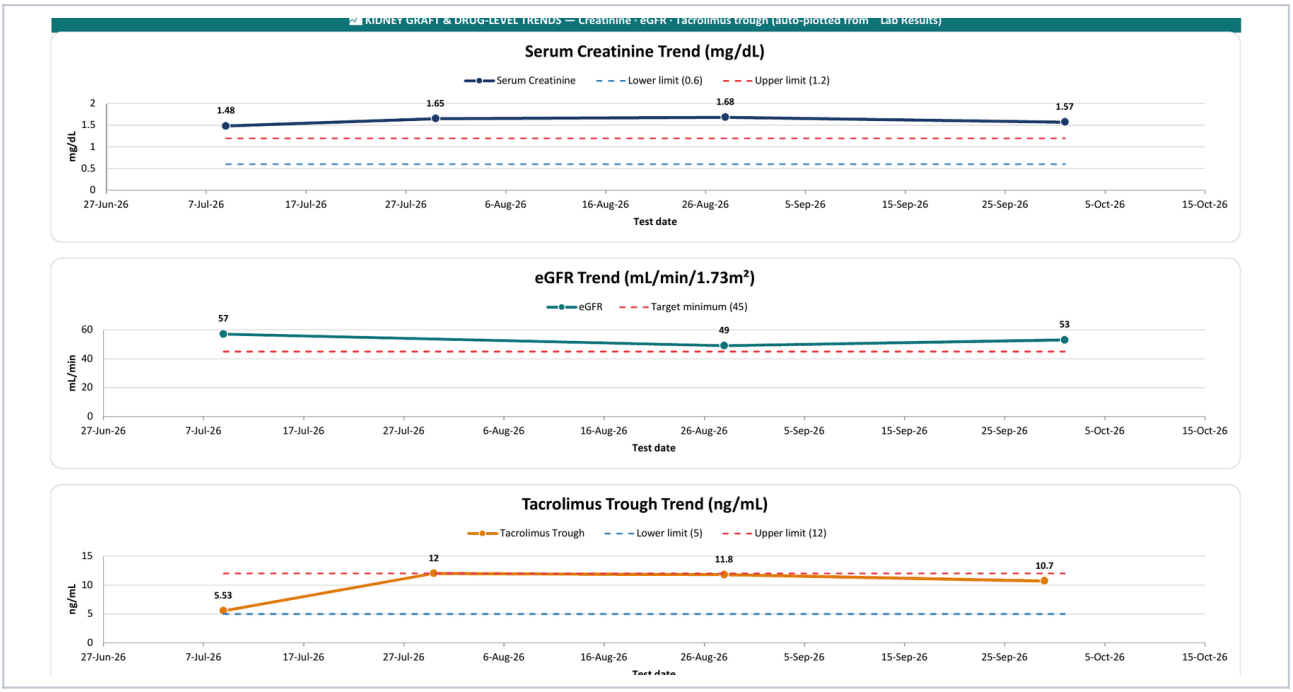


Figure 9 Kidney graft and drug-level trends

Three charts share the same date axis. Dashed lines show the lower and upper limits (or the target minimum for eGFR) taken from Dashboard Settings. Each marker is one lab test date, labelled with its value.

2. Visual Layout Blueprint

Map of the two Dashboard pages

```
┌──────────────────────────────────────────────────────────────────────────────────────┐
│ CLINICAL TRENDS DASHBOARD - AUTO-GENERATED FROM DAILY VITALS & LAB RESULTS      [PAGE 1]
├──────────────────────────────────────────────────────────────────────────────────────┤
│ AUTOMATIC CLINICAL ALERTS  (red bar)
│   Latest BP | Latest SpO2 | Latest Blood Sugar | Weight 7d change | Latest Creatinine |
│   Latest Tacrolimus | Labs out of range            (7 tiles, each one line of plain English)
├──────────────────────────────────────────────────────────────────────────────────────┤
│ KIDNEY GRAFT & DRUG-LEVEL TRENDS  (teal bar)
│   [ Chart 1: Serum Creatinine + limits ]   [ Chart 2: eGFR + target ]   [ Chart 3: Tacrolimus + limits ]
├──────────────────────────────────────────────────────────────────────────────────────┤
│ LAB SNAPSHOT - latest result vs previous, checked against your set limits
│   Parameter | Unit | Latest | Date | Previous | Change | Trend | Status | Readings | Frequency
│   (23 numeric tests)   then   Qualitative / PCR tests (BKV, CMV, Urine RE, Urine C/S)
├──────────────────────────────────────────────────────────────────────────────────────┤
│ DAILY HOME VITALS  (teal bar)                                                [PAGE 2]
│   [ Weight chart ]  [ Urine Output chart ]  [ Blood Pressure chart ]  [ SpO2 chart ]
├──────────────────────────────────────────────────────────────────────────────────────┤
│ FASTING BLOOD SUGAR - 30-day summary and trend
│   [Latest][7-day avg][30-day avg][% in target][>180][<70][Readings][Latest HbA1c]   + chart
├──────────────────────────────────────────────────────────────────────────────────────┤
│ END OF DASHBOARD  (navy bar)
└──────────────────────────────────────────────────────────────────────────────────────┘
```

LAB SNAPSHOT — latest result vs previous, checked against your set limits

Latest lab date on file: 01-Oct-2026 | Parameters with results: 20 of 23 | Outside limits: 7

Parameter	Unit	Latest	Date	Previous	Change	Trend	Normal range	Post-Tx target	Status	Readings	Frequency
Serum Creatinine	mg/dL	1.57	1-Oct-26	1.68	-0.11	▼ Falling	0.6 – 1.2	Stable baseline	▲ HIGH	4	Monthly
eGFR	mL/min	53	1-Oct-26	49	+4.0	▲ Rising	≥ 60	≥ 45	✓ In range	3	Monthly
Tacrolimus Trough	ng/mL	10.7	29-Sep-26	11.8	-1.1	▼ Falling	—	Early: 8–12 | Late: 5–8	✓ In range	4	Monthly
Calcium (Total)	mg/dL	8.1	1-Oct-26	9.5	-1.4	▼ Falling	8.5 – 10.5	8.5 – 10.2	▼ LOW	3	Monthly
Phosphorus	mg/dL	3	1-Oct-26	3	0	• Stable	2.5 – 4.5	2.5 – 4.5	✓ In range	3	Monthly
Uric Acid	mg/dL	5.8	1-Oct-26	5.5	+0.3	▲ Rising	2.4 – 7.0	< 7.0	✓ In range	3	Monthly
Potassium	mEq/L	4.3	1-Oct-26	3.9	+0.4	▲ Rising	3.5 – 5.0	3.5 – 5.0	✓ In range	3	Monthly
Sodium	mEq/L	141	1-Oct-26	139	+2.0	• Stable	135 – 145	135 – 145	✓ In range	3	Monthly
Serum Bicarbonate	mEq/L	—	—	—	—	—	22 – 29	22 – 29	—	0	Monthly
FBS (Fasting Sugar)	mg/dL	145.9	30-Jul-26	—	—	—	70 – 100	70 – 126	▲ HIGH	1	Monthly
PPBS (Post-meal)	mg/dL	205.44	30-Jul-26	—	—	—	≤ 140	≤ 180	▲ HIGH	1	Monthly
HbA1c	%	7.5	28-Aug-26	—	—	—	≤ 5.7	≤ 7.0	▲ HIGH	1	3-Monthly
Haemoglobin	g/dL	13.6	1-Oct-26	12.8	+0.8	▲ Rising	12 – 17	11 – 14	✓ In range	3	Monthly
TLC (WBC)	×10³/µL	7.2	1-Oct-26	9.9	-2.7	▼ Falling	4 – 11	4 – 10	✓ In range	3	Monthly
Platelets	×10³/µL	279	1-Oct-26	318	-39.0	▼ Falling	150 – 400	150 – 350	✓ In range	3	Monthly
Serum Albumin	g/dL	3.8	1-Oct-26	3.7	+0.1	• Stable	3.5 – 5.0	3.5 – 5.0	✓ In range	3	Monthly
Total Bilirubin	mg/dL	0.5	1-Oct-26	—	—	—	≤ 1.2	≤ 1.2	✓ In range	1	3-Monthly
SGOT / AST	U/L	41	1-Oct-26	—	—	—	≤ 40	≤ 40	▲ HIGH	1	3-Monthly
SGPT / ALT	U/L	65	1-Oct-26	—	—	—	≤ 56	≤ 56	▲ HIGH	1	3-Monthly
Lipid Profile (LDL)	mg/dL	81	1-Oct-26	92.2	-11.2	▼ Falling	≤ 100	≤ 100	✓ In range	2	3-Monthly
Vitamin D (25-OH)	ng/mL	—	—	—	—	—	30 – 100	30 – 100	—	0	3-Monthly
Urine PCR (UACR)	mg/g	33.3	1-Oct-26	24.1	+9.2	▲ Rising	< 150	< 300	✓ In range	3	Monthly
24h Urine Protein	mg/24h	—	—	—	—	—	< 150	< 300	—	0	If indicated

Qualitative / PCR tests	Unit	Latest result	Date	Previous result	Post-Tx target	Status	Frequency
BKV PCR	copies/mL	Below Detection Limit	30-Jul-26	—	< 10,000	Doctor review	Monthly × 9m then 3m
CMV PCR	copies/mL	—	—	—	Undetectable	—	Monthly × 6m
Urine RE (Protein)	–	Not Detected (Nil)	28-Aug-26	Absent (Nil)	Nil / Trace	Doctor review	Monthly
Urine RE (RBC/WBC)	cells/hpf	0 RBC / 0 WBC	28-Aug-26	0 RBC / 4 WBC	≤ 5	Doctor review	Monthly
Urine C/S	–	—	—	—	No Growth	—	If symptomatic

How to read: Status compares the latest value with the Low / High limits in ⚙ Dashboard Settings (editable). Trend compares the latest with the previous reading. The Date column shows when each value was last tested - older values may be out of date. Enter lab dates oldest to newest, left to right, in Lab Results (out-of-order dates are blocked). This is a decision-support summary only; the treating doctor interprets the results.

Figure 10 Lab Snapshot — latest result against previous, checked against your limits

Columns show the unit, latest value and its date, the previous value, the change, a trend arrow, a coloured Status, the number of readings and the usual testing frequency. A second table below lists text-based tests such as BKV PCR and urine routine examination for the doctor to review.

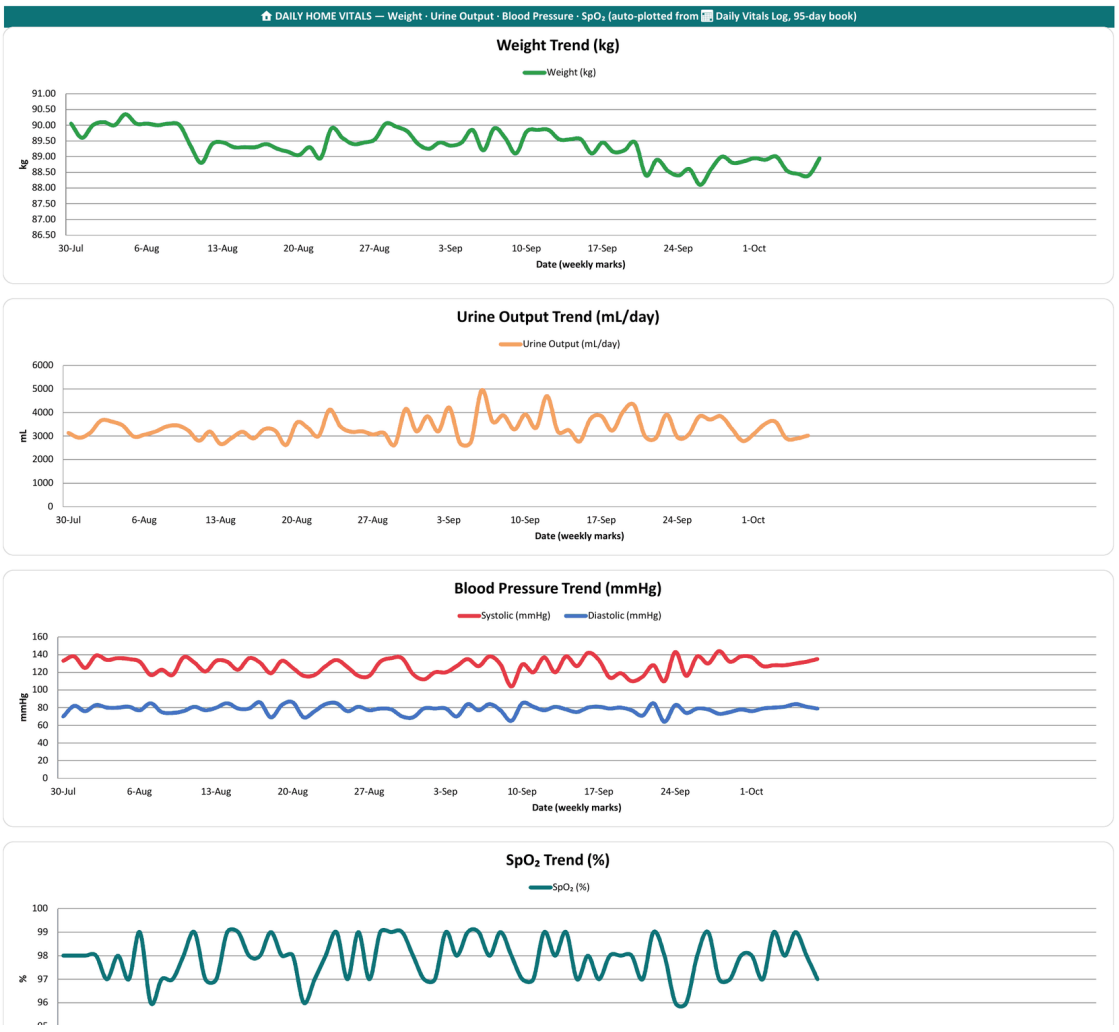


Figure 11 Daily home vitals charts (page 2)

Weight, urine output, blood pressure (systolic and diastolic) and SpO_2 are plotted day by day from the Daily Vitals Log, with weekly date marks along the bottom.

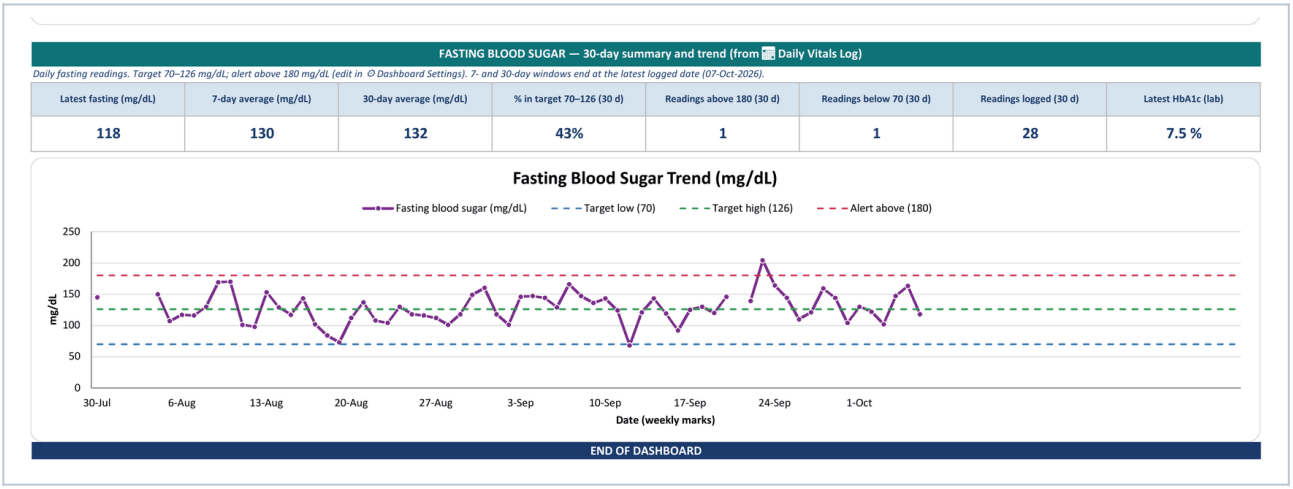


Figure 12 Fasting blood-sugar summary and trend

Eight summary tiles (latest, 7-day and 30-day average, percentage in target, readings above and below limits, readings logged, latest HbA1c) sit above a trend chart with the target band and the alert line. A navy bar marks the END OF DASHBOARD.

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Red	**Red bar — AUTOMATIC CLINICAL ALERTS**	Heading of the alert block.
Aa 123	**▲ HIGH (red fill)**	The latest value is **above** your high limit. Inform your doctor.
Aa 123	**▼ LOW (amber fill)**	The latest value is **below** your low limit.
Aa 123	**✔ In range (green fill)**	The latest value is within your limits.
Teal	**Teal bars**	Chapter-style headings for each block (kidney graft trends, lab snapshot, home vitals, blood sugar).
Navy	**Navy bar**	Table headings and the END OF DASHBOARD line.

How the alert tiles decide (the default limits can be edited in Chapter 11):

Tile	Flags a warning when...
Latest BP	Latest systolic is above 140 or below 90, or diastolic is above 90.
Latest SpO_2	Latest value is below 94%.
Latest Blood Sugar	Latest fasting sugar is above 180 mg/dL.
Weight 7d change	The latest weight is more than 2 kg above the **average of the previous 7 days** (possible fluid overload).
Latest Creatinine	**Borderline** above 1.5 mg/dL; **urgent** above 2.0 mg/dL.
Latest Tacrolimus	Below 5 or above 12 ng/mL.
Labs out of range	Counts the lab parameters whose latest value is outside its limits, and lists them.

4. Data Input Requirements

There is nothing to type on the Dashboard. Treat every cell as read-only.

- To change what appears here, change the **source**: add rows in the **Daily Vitals Log**, add test dates in **Lab Results**, or adjust limits in **Dashboard Settings**.
- Charts refresh by themselves. If a chart looks empty, check that dates in the source sheets are real dates and that results are numbers.

> **HOW MANY DAYS DO THE CHARTS SHOW?**
> The four daily-vitals charts plot the **first 95 rows** of the Daily Vitals Log. The alert tiles, sugar summaries and weekly figures use every row (up to 110). The three lab charts show up to the 20 test dates of Lab Results.

> **OLD VALUES ARE LABELLED WITH THEIR DATE**
> The Lab Snapshot's **Date** column shows when each test was last done. An old value may no longer describe today's health — look at the date before you trust the Status.

5. Lifecycle & Triggers

When	What to do
Every few days	Glance at the alert block — are all tiles ✅?
After entering new labs	Check the lab charts and the snapshot for ▲ / ▼ statuses.
Before every visit	Print or save the two Dashboard pages as part of the Doctor Pack.
You never "start" this sheet	It builds itself from your other sheets.

CHAPTER 7 · AUTOMATIC SHEET + DOCTOR'S PLAN ★ DOCTOR PACK

Chapter 7: Doctor Referral Report

One page that tells your doctor who you are, how you are, and what was decided.

1. Purpose & Requirement

In a 10–15 minute consultation, a doctor needs the headline first and the detail second. The **Doctor Referral Report** is that headline: a single printed page assembled automatically from your other sheets. Its lower part is deliberately left for the **doctor's own notes, illness-management plan, tests required and next visit date**.

- **Identity block** from Patient Info; **doctor / hospital line** at the top.
- **Vitals summary, last 7 days:** minimum, maximum and average of BP, pulse, SpO_2, weight, fluids, urine and blood sugar, each with a clinical note.
- **Latest lab results:** the most recent value of 16 key tests, with unit, post-transplant target and notes.
- **Current medications:** the ACTIVE medicines from Medication History (up to 15) with a heading count.
- **Doctor's section:** notes, plan, tests required and next visit date.

CLINICAL SUMMARY FOR TREATING NEPHROLOGIST — KIDNEY TRANSPLANT FOLLOW-UP

Auto-populated from all sheets | Print & send to doctor before visit | Do not edit — data pulls automatically

		Reg:	Phone:

PATIENT IDENTIFICATION

Patient Name	
Patient ID	
Date of Birth	
Gender	Male
Blood Group	O+ve
Transplant Date	25-Apr-2026
Diagnosis	DM T2, Hypertension, Obesity, CKD (T)
Comorbidities	DM T2, Hypertension, Obesity

VITALS SUMMARY — LAST 7 DAYS

Parameter	Min	Max	Average	Unit	Clinical Note
BP Systolic (mmHg)	127.0	135.0	130.0	mmHg	Target: <130/80
BP Diastolic (mmHg)	79.0	84.0	80.7	mmHg	Target: <80
Pulse (bpm)	70.0	85.0	80.3	bpm	Normal: 60–100
SpO_2 (%)	97.0	99.0	98.0	%	Alert if <94%
Weight (kg)	88.4	89.0	88.7	kg	Gain >2 kg/week = alert
Fluid Intake (mL/day)	3000.0	3600.0	3240.0	mL	Target: ≥4000 mL/day
Urine Output (mL/day)	2900.0	3610.0	3186.0	mL	Target: ≥1000 mL/day
Blood Sugar (mg/dL)	102.0	163.0	130.0	mg/dL	>180 mg/dL = alert (limit set in Dashboard Settings)

LATEST LAB RESULTS (from Lab Results sheet — most recent of up to 20 test dates)

Test	Latest Result	Unit	TX Target	Notes
Creatinine	1.57	mg/dL	Stable baseline	Rising trend = rejection risk
eGFR	53	mL/min	≥45	
Tacrolimus Trough	10.7	ng/mL	Early:8–12 | Late:5–8	
Calcium	8.1	mg/dL	8.5–10.2	
Phosphorus	3	mg/dL	2.5–4.5	
Uric Acid	5.8	mg/dL	<7.0	
Potassium	4.3	mEq/L	3.5–5.0	
FBS	145.9	mg/dL	70–126	>180 = PTDM
PPBS	205.44	mg/dL	≤180	
HbA1c	7.5	%	≤7.0	
Haemoglobin	13.6	g/dL	11–14	
TLC (WBC)	7.2	$\times 10^3/\mu L$	4–10	
Platelets	279	$\times 10^3/\mu L$	150–350	
BKV PCR	Below Detection Limit	copies/mL	<10,000	
Urine RE Protein	Not Detected (Nil)	-	Nil/Trace	
Urine PCR (UACR)	33.3	mg/g	<300	

Figure 13 Doctor Referral Report — identity, 7-day vitals and latest labs

The grey-blue bars hide the patient's name, ID, date of birth and the doctor line. All values are pulled automatically — the sheet is marked "Do not edit".

CURRENT MEDICATIONS — 10 active (Status = ACTIVE in Medication History)					
No. · Brand Name	Composition	Category	Dose	Timing	Notes
1. Mycept S 360mg (Mycophenolate Na)	Mycophenolate Sodium 360 mg	Immunosuppressant (MPA)	360 mg	1-0-1 (After Food)	Anti-rejection; monitor TLC — leukopenia risk
2. Wysolone 5mg (Prednisolone)	Prednisolone 5 mg	Corticosteroid	5 mg	1-0-0 (After Food)	Morning dose; monitor blood sugar — PTDM risk
3. Vagacyte 450mg (Valganciclovir)	Valganciclovir 450 mg	Antiviral Prophylaxis (CMV)	450 mg	0-0-1 (After Food)	CMV prophylaxis; monitor CBC
4. Lupinsulin 30:70 Injection	Human Insulin 30% + NPH Human Insulin 70%	Antidiabetic (Insulin)	38U-0-22U	Before Food - Daily	20-30 min before meals. Store in fridge 2-8°C
5. Eurepa V 1/0.3mg Tablet	Repaglinide (1 mg) + Voglibose (0.3 mg)	Antidiabetic	1 Tablet	Twice a day (After Lunch & Dinner)	Take immediately before or up to 15-30 min before Meal. Prevents sharp BS spikes after meals.
6. Dilzem CD 90 mg Capsule	Diltiazem Hydrochloride (90 mg)	Antihypertensive / Calcium Channel Blocker	1 Tablet	Once a day (Morning)	After breakfast. Do not crush or chew the ER capsule.
7. Nebicard 2.5mg	Nebivolol (2.5 mg)	Antihypertensive / Beta-Blocker	1 Capsule	Once a day (Night)	After dinner. Helps manage BP and heart rate.
8. GLUCRETA M 10 / 500MG TABLET	Dapagliflozin 10 MG + Metformin 500 MG	An Anti-diabetic combination	1 Tablet	Once a day (Morning)	After breakfast. Do not crush or chew the Tablet.
9. Pangraf 2.25 mg (Tacrolimus)	Tacrolimus 2+0.25 mg	Immunosuppressant (CNI)	2.25 mg	1-0-1 (After Food)	Dose guided by trough level (Tac C0); check renal function [Dose Decr]
10. Wegovy 2.4MG FLEXTOUCH 3ML	Semaglutide 2.4 MG [0.5 mg/week subcutaneous]	Antidiabetic / Anti-obesity (Incretin mimetic)	0.5 mg / Week	Weekly	Dose Revised to 0.5mg/wk. SC.

NOTES / ILLNESS MANAGEMENT PLAN / LAB TESTS REQUIRED FOR NEXT FOLLOW-UP (to be filled in during consultation)	Next Visit Date : Fri, 04-Dec-2026		
	4	Dec	2026
NOTES (History · Examination · Diagnosis)	**Diagnoses** : 1. Diabetes Mellitus - II, 2. Hypertension, 3. Obesity, and 4. CKD (T) (Chronic Kidney Disease)		
	General History • **Urinary Status**: No history of change in Urine Output (UOP) or Lower Urinary Tract Symptoms (LUTS). • **Systemic Symptoms**: No history of fever, loin pain, general malaise, nausea, or vomiting. • **Physical Signs**: No history of hand tremors, changes in facial features, or new skin lesions/masses. • **Oral & Gastrointestinal**: No history of oral thrush, painful swallowing (odynophagia), or loose stools. • **Musculoskeletal**: No history of limb pains. • **Medication & Hospitalization**: No history of drug non-compliance or recent hospitalizations outside of the current nephrologist's care for previous issues.		
	Examination Status : Seen online (teleconsultation).		
ILLNESS MANAGEMENT PLAN (Revised medication directions · Diet · Exercise · Advice)	**Medications**: Strictly follow the 10 listed regimens, tracking split timings (After Food for pills; Lupinsulin 20–30 mins Before Food, skip if skipping meals; Wegovy weekly before lunch).		
	Diet: Manage diabetes and obesity while protecting your kidneys by balancing nutritional intake around your upcoming Calcium, Potassium, and Phosphorus lab parameters.		
	Exercise: Actively integrate the doctor's specific recommendation for muscle strengthening exercises into your regular physical routine.		
	Advice: Log daily blood pressure/glucose, complete all 13 listed monitoring labs (including Tac Level C0, Creatinine, and HbA1c), and follow up on 04-Dec-2026.		
LAB TESTS REQUIRED (to be done before the next Follow-up)	**Tests Scheduled**: (Multiple monitoring labs) : TAC Level C0 from [redacted]		
	BKV Quantitative Blood, Creatinine, Calcium, Phosphorous, Uric Acid, Potassium, FBS, PPBS, CBC Count, HbA1c, Lipid Profile		
	Urine RE, UACR		

Figure 14 Doctor Referral Report — current medicines and the doctor's section

Below the active-medicine list is the area for the consultation: Notes (history, examination, diagnosis), Illness Management Plan and Lab Tests Required, each with four yellow lines, and a Next Visit Date chosen with three dropdowns.

2. Visual Layout Blueprint

Map of the Doctor Referral Report (one page)

```
┌──────────────────────────────────────────────────────────────────────────┐
│ CLINICAL SUMMARY FOR TREATING NEPHROLOGIST - KIDNEY TRANSPLANT FOLLOW-UP       [title]          │
│ Auto-populated from all sheets | Print & send to doctor before visit | Do not edit              │
│ Doctor name | Hospital | Reg: ... | Phone: ...                                    [teal line] │
├──────────────────────────────────────────────────────────────────────────┤
│ PATIENT IDENTIFICATION   Name | ID | Date of Birth | Gender | Blood Group | Transplant Date |  │
│                          Diagnosis | Comorbidities                    (from Patient Info)     │
├──────────────────────────────────────────────────────────────────────────┤
│ VITALS SUMMARY - LAST 7 DAYS   Parameter | Min | Max | Average | Unit | Clinical Note          │
│   BP Systolic, BP Diastolic, Pulse, SpO2, Weight, Fluid Intake, Urine Output, Blood Sugar      │
├──────────────────────────────────────────────────────────────────────────┤
│ LATEST LAB RESULTS   Test | Latest Result | Unit | TX Target | Notes      (16 key tests)      │
├──────────────────────────────────────────────────────────────────────────┤
│ CURRENT MEDICATIONS - n active (Status = ACTIVE)   No.+Brand | Composition | Category | Dose |  │
│   Timing | Notes                                          (room for 15 medicines)             │
├──────────────────────────────────────────────────────────────────────────┤
│ NOTES / ILLNESS MANAGEMENT PLAN / LAB TESTS REQUIRED FOR NEXT FOLLOW-UP   | Next Visit Date :  │
│   NOTES (History - Exam - Diagnosis)        [4 yellow lines]      | [DD] [MMM] [YYYY] dropdowns │
│   ILLNESS MANAGEMENT PLAN                   [4 yellow lines]                                   │
│   LAB TESTS REQUIRED                        [4 yellow lines]                                   │
└──────────────────────────────────────────────────────────────────────────┘
```

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Navy	**Navy bars**	Section headings: Patient Identification, Vitals Summary, Latest Lab Results, Current Medications.
Teal	**Teal line**	Doctor name, hospital, registration and phone — copied from Patient Info.
Blue	**Blue table headers**	Column titles of the vitals, lab and medicine tables.
Aa 123	**Pale-blue label cells**	Row labels such as "Patient Name". Fixed.
Aa 123	**Pale-yellow lines**	**The only cells that are written on**: the doctor's notes, plan and tests lines.
Aa 123	**White cells**	Automatic values. Do not overwrite.

4. Data Input Requirements

Everything above the doctor's section is automatic. In the doctor's section, the following may be typed (or written by hand on the printed copy):

- **Notes (History · Examination · Diagnosis)** — four lines, free text.
- **Illness Management Plan** (revised medicine directions, diet, exercise, advice) — four lines, free text.
- **Lab Tests Required** (to be done before the next follow-up) — four lines, free text; list the tests one per line or separated by commas.
- **Next Visit Date** — choose **day** (1–31), **month** (Jan–Dec) and **year** (2026–2035) from the three yellow dropdowns. When all three are chosen, the heading shows the full date with the weekday, for example *Next Visit Date : Fri, 04-Dec-2026*. If you pick an impossible date, such as 31 Feb, it says "invalid date — please re-check".

> ⚠ **"LAST 7 DAYS" COUNTS BACK FROM TODAY**
> The vitals table looks at the last seven days **before the date on your computer**. If you did not log anything recently, the cells show "—". Update the Daily Vitals Log first, then print.

> ★ **DROPDOWN ARROWS**
> Click a yellow date cell and a small arrow appears at its right edge. If the arrow is missing after scrolling sideways, scroll back to the left (press `Ctrl+Home`).

5. Lifecycle & Triggers

When	What to do
Before every visit	Refresh your data, then print or save this page as part of the Doctor Pack.
During the consultation	The doctor writes notes, plan, tests and the next visit date.
After the visit	Type the doctor's notes (if written by hand), set the Next Visit Date, then update Medication History and the Doctor Visit Log.
Next cycle	Clear the yellow lines and reset the date dropdowns to DD / MMM / YYYY.

CHAPTER 8 · AUTOMATIC SHEET · 1 PAGE

Chapter 8: Weekly Summary

Week-by-week averages that smooth out the daily ups and downs.

1. Purpose & Requirement

A single day's blood pressure can mislead; a week's average tells the truth. The **Weekly Summary** groups your Daily Vitals Log into Monday-to-Sunday weeks and calculates averages and alert counts for each week.

- **Metrics compiled:** average systolic and diastolic BP, pulse, SpO_2, weight, fluid in, urine out and blood sugar; **net fluid balance** for the week; number of **SpO_2 alert days** and **BP alert days**.
- **Weeks appear by themselves:** the first Monday is taken from the first date in your log and rows continue to the last logged date.
- **Not part of the Doctor Pack** — keep it ready if the doctor asks for week-by-week averages.

WEEKLY SUMMARY — AUTO-CALCULATED FROM DAILY VITALS LOG

Auto-generated from Daily Vitals Log. Weeks are generated automatically from the first date in the log (Monday-based) and stop at the last logged date.

HOW TO USE: Nothing to type in column B — week-start (Monday) dates fill in by themselves from the Daily Vitals Log. Averages and alert counts for each 7-day period are calculated automatically. The first week may be partial if the log starts mid-week. Do not edit columns B–N.

Week Start (Mon Date)	Week End (+6 days)	Avg Systolic (mmHg)	Avg Diastolic (mmHg)	Avg Pulse (bpm)	Avg SpO_2 (%)	Avg Weight (kg)	Avg Fluid In (mL/day)	Avg Urine Out (mL/day)	Avg Blood Sugar (mg/dL)	Net Balance (mL/week)	SpO_2 Alert Days	BP Alert Days	Notes
27-Jul-2026	02-Aug-2026	133.8	77.8	87.0	98.0	89.94	3500	3215	145	1140	0	0	
03-Aug-2026	09-Aug-2026	127.7	78.9	85.1	97.3	90.07	3357	3304	132	370	0	0	
10-Aug-2026	16-Aug-2026	130.4	79.6	83.3	98.1	89.27	3071	2984	130	610	0	0	
17-Aug-2026	23-Aug-2026	124.0	79.0	77.9	97.7	89.29	3343	3311	103	220	0	0	
24-Aug-2026	30-Aug-2026	128.0	78.0	80.0	98.4	89.69	3329	3253	121	530	0	0	
31-Aug-2026	06-Sep-2026	122.7	76.7	80.9	98.1	89.42	3557	3550	135	50	0	0	
07-Sep-2026	13-Sep-2026	125.3	78.6	78.7	98.0	89.66	3714	3704	129	70	0	0	
14-Sep-2026	20-Sep-2026	126.3	78.6	85.0	97.9	89.35	3643	3599	125	310	0	1	
21-Sep-2026	27-Sep-2026	125.7	76.3	86.3	97.6	88.51	3371	3336	147	250	0	1	
28-Sep-2026	04-Oct-2026	133.4	77.4	84.0	97.7	88.86	3329	3290	130	270	0	1	
05-Oct-2026	11-Oct-2026	132.3	81.3	76.3	98.0	88.60	3050	2960	141	180	0	0	

Figure 15 The Weekly Summary

Each row is one week. Averages are rounded sensibly (BP to one decimal, weight to two, fluids to whole millilitres). A red cell appears when SpO_2 alert days or BP alert days cross the warning threshold.

2. Visual Layout Blueprint

Map of the Weekly Summary

```
┌──────────────────────────────────────────────────────────────────────────────────────────────┐
│ WEEKLY SUMMARY - AUTO-CALCULATED FROM DAILY VITALS LOG                          [title bar]  │
│ HOW TO USE: nothing to type in column B - week-start (Monday) dates fill in by themselves    │
├──────────────────────────────────────────────────────────────────────────────────────────────┤
│ Week     Week     Avg    Avg    Avg    Avg    Avg     Avg      Avg      Avg     Net      SpO2   BP                │
│ Start    End      Sys    Dia    Pulse  SpO2   Weight  Fluid In Urine    Sugar   Balance  Alert  Alert  Notes      │
│ (Mon)    (+6 d)                                              Out              mL/wk    Days   Days               │
├──────────────────────────────────────────────────────────────────────────────────────────────┤
│ 27-Jul   02-Aug   133.8  77.8   87.0   98.0   89.94   3500     3215     145     1140     0      0                 │
│ 03-Aug   09-Aug   127.7  78.9   85.1   97.3   90.07   3357     3304     132     370      0      0                 │
│    :   one row per week - generated automatically, up to the last logged date                                     │
├──────────────────────────────────────────────────────────────────────────────────────────────┤
│ DOCTOR'S INTERPRETATION GUIDE   (weight trend | BP alerts | net fluid balance | SpO2 | sugar)                    │
└──────────────────────────────────────────────────────────────────────────────────────────────┘
```

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Aa 123	**Red cell — SpO_2 Alert Days**	One or more days in that week had SpO_2 below 94%. **Prompt review needed.**
Aa 123	**Red cell — BP Alert Days**	More than three days in that week had BP outside the limits. Discuss with your doctor.
Aa 123	**White cell**	Averages and counts — automatic.
Navy	**Navy header**	Column titles and units.

4. Data Input Requirements

Nothing to type. The sheet says "Do not edit columns B–N". The Notes column may be used for your own remarks if you wish.

The doctor's interpretation guide printed on the sheet reads, in plain words:

- **Weight trend:** a gain of more than 1 kg in two days, or more than 2 kg in a week, may mean fluid overload.
- **BP alerts:** many out-of-range days → the doctor may review blood-pressure medicines.
- **Net fluid balance:** more than +500 mL per week → check for swelling and graft function.
- **SpO_2 alerts:** any red day requires prompt review (fluid in the lungs or infection).
- **Average sugar:** consistently above your alert limit → review for post-transplant diabetes (PTDM).

5. Lifecycle & Triggers

When	What to do
Once a week (suggested: Monday)	Read last week's row; note any red cell.
Before a visit, if asked	Print this one page.
New quarterly workbook	Starts again automatically from the first date you log.

CHAPTER 9 · READOUT + RECORD SHEET · EACH VISIT

Chapter 9: Doctor Visit Log

A permanent record of every consultation, filled in three easy moves.

1. Purpose & Requirement

The **Doctor Visit Log** keeps one row for **each consultation**, holding the key numbers of that day. Months later, you can run your eye down a column and see how weight, creatinine or tacrolimus moved from visit to visit — without searching through old reports.

- **Metrics compiled per visit:** date; fasting sugar; BP (systolic/diastolic); pulse; weight; calculated BMI; creatinine, eGFR, Hb, TLC, platelets, urine PCR, tacrolimus level, BKV PCR; tacrolimus dose; other immunosuppressants; doctor's notes; next consultation date.
- **A smart readout row** fetches the vitals and labs for any date you pick, so you do not retype numbers.
- **A visit-schedule guide** suggests the date of the next visit from your transplant date.
- **A one-time HEIGHT cell** makes the BMI calculation possible.

DOCTOR CONSULTATION / FOLLOW-UP VISIT LOG — POST-TRANSPLANT NEPHROLOGY

HOW TO USE: ① Pick the consultation date in B5. ② Copy the pink cells C4:P4. ③ Type the same date in column B of the next blank row, then PASTE SPECIAL • VALUES (Ctrl+Alt+V, then V) into C of that row. ④ Fill the white columns Q–T. (A cell turns ORANGE if a formula was pasted by mistake — re-paste as Values.)

⚠ ONE-TIME SETUP — enter the patient's HEIGHT (cm) in the yellow box first, before recording any visit. Enter it ONCE and do NOT change it later: every BMI in this log is calculated from it.

HEIGHT (cm) ► 173

✓ Height fixed at 173 cm — do not change

Date of Consultation	FBS (mg/dL)	BP-Sys (mmHg)	BP-Dias (mmHg)	Pulse (bpm)	Weight (kg)	Calculated BMI	Creatinine (mg/dL)	eGFR (mL/min)	Hb (g/dL)	TLC (×10³/µL)	Platelets (×10³/µL)	Urine PCR (mg/g)	Tac Level (ng/mL)	BKV PCR (copies/mL)	Tac Dose (mg/day)	Other Immuno Suppressants	Doctor's Notes / Action Plan	Next Consultation Date
Consltn Date ↓	147	128	81	83	88.55	29.59	1.57	53	13.6	7.2	279	33.3	10.7	Below Detection Limit		Schedule guide ►	Post-Tx 5 m 9 d • Bi-monthly visits (60 d)	03-Dec-2026
04-Oct-2026	Data from : Daily Vitals Log					Formulae Based	Data from : Lab Results — latest available result on / before the selected date (newest lab date used: 01-Oct-2026) • DARKER pink = older result carried forward								Manual Input			
05-Jun-2026	121	153	82	87	93.95	31.39	1.62		9.9	5.6	420	15.99	8.54		4+4 mg	Mycept, Vagacyte	At [redacted]	15-Jun-2026
27-Jun-2026	187	145	84	93	91.3	30.51	1.69	49	10.6	9.34	378	215	7.71		2.5+2.5	Mycept, Vagacyte	Online Consln; Meds Revised	18-Jul-2026
31-Jul-2026	145	138	82	82	89.6	30	1.65	49	11.5	10.9	325		12		2.5+2.5	Mycept, Vagacyte	Online Consln; Meds Revised	31-Aug-2026
29-Aug-2026	118	136	78	78	89.95	30.1	1.68	49	12.8	9.9	318	24.1	11.8		2.5+2.5	Mycept, Vagacyte	Online Consln; Meds Revised	04-Oct-2026
04-Oct-2026	147	128	81	83	88.55	29.59	1.57	53	13.6	7.2	279	33.3	10.7		2+2 mg	Mycept, Vagacyte	Online Consln; Meds Revised	04-Dec-2026

Figure 16 The Doctor Visit Log (one name hidden in the notes column)

Top: how-to note, the HEIGHT setup cell and its status message. Row 4 (pink) is the automatic readout for the date in B5; its green strip shows the schedule guide and a suggested next date. The five rows below are saved consultations, each with its manual entries in the white columns on the right.

2. Visual Layout Blueprint

Map of the Doctor Visit Log

```
DOCTOR CONSULTATION / FOLLOW-UP VISIT LOG - POST-TRANSPLANT NEPHROLOGY          [row 1]
HOW TO USE (steps 1-4) | ONE-TIME SETUP: enter HEIGHT -> | HEIGHT (cm) [ P2 ] | status message  [row 2]
Date of | FBS | BP-Sys | BP-Dias | Pulse | Weight | BMI | Creat | eGFR | Hb | TLC | Plt | UPCR | Tac |
Consult.| ... (B3 to P3 headers)  ...                  | BKV | Tac Dose | Other Immuno | Notes | Next Date  [row 3]
Consltn Date v | PINK READOUT  C4:P4  (automatic)  | green strip Q4:T4 (schedule guide, suggested date) [row 4]
[ B5 ] pick date | Data from: Daily Vitals Log | BMI | Data from: Lab Results | MANUAL INPUT (black bar) [row 5]
05-Jun-2026 | 121 | 153 | 82 | 87 | 93.95 | 31.39 | 1.62 | ... | 4+4 mg | Mycept | At CMRI | 15-Jun-2026
27-Jun-2026 | ...        one row per consultation  (rows 6 to 55 = 50 visits)
```

3. Color-Coded Guidance

This sheet uses its own colour scheme, designed for the new workflow.

Colour	What you see	What it means / what to do
Aa 123	**Pink — automatic readout (C4:P4)**	Values fetched for the date in B5. **Copy these**, but never type in them.
Aa 123	**Darker pink, italic**	That value is **older**: the newest lab report did not include this test, so the latest earlier result is shown instead.
Aa 123	**Green**	The date selector (B5) and the green strip Q4:T4 (schedule guide, suggested next date).
Aa 123	**Blue band (row 5)**	"Data from: Daily Vitals Log" — columns C to G.
Aa 123	**Salmon**	Formula-based: BMI (column H).
Aa 123	**Grey band**	"Data from: Lab Results" — columns I to P. The band also states the newest lab date used.
Manual	**Black bar "Manual Input"**	Columns Q to T are filled by hand after each visit.
Aa 123	**Pale-yellow (column B)**	The consultation date of a saved row.
Aa 123	**White (Q to T)**	Manual entries: Tac Dose, Other Immunosuppressants, Doctor's Notes, Next Consultation Date.
Aa 123	**Orange warning**	A saved cell that still contains a **formula** (pasted without "Values"). Re-paste it as values.
Aa 123	**Yellow HEIGHT box with red border**	Enter the patient's height here. Red message "HEIGHT MISSING" while empty; green "Height fixed at … cm" once filled.

4. Data Input Requirements

Set up once

- **HEIGHT (cm)** (`P2`) — a number between 100 and 230, for example `173`. Enter it **before** recording any visit and **never change it later**: every BMI in the log uses it.

For each visit — the three-move routine

RECORDING A CONSULTATION

1. **Pick the date.** Click `B5` and choose the consultation date from the dropdown (newest dates first), or type it as `dd-mmm-yyyy`. The pink readout fills itself.
2. **Copy and paste values.** Select `C4:P4` and copy. In the next **blank row**, type the same date in column **B**, click the cell in column **C** of that row, and use **Paste Special ▸ Values** (`Ctrl+Alt+V`, then `V`).
3. **Complete the white columns.** Type the **Tac Dose**, **Other Immunosuppressants**, **Doctor's Notes / Action Plan** and **Next Consultation Date** (`dd-mmm-yyyy`).

ALWAYS PASTE VALUES

A normal paste copies the **formulas**. The new row would then change every time you pick another date in B5 and your saved record would be overwritten. If a pasted cell turns **orange**, it still holds a formula: copy the cell again and paste it as **Values**.

- **Tac Dose** (`Q`) — text such as `2+2 mg`.
- **Other Immuno Suppressants** (`R`) — text, for example *Mycophenolate, Valganciclovir*.
- **Doctor's Notes / Action Plan** (`S`) — short text.
- **Next Consultation Date** (`T`) — a date; type exactly as the doctor advised. The green strip's suggestion is only a guide.

The schedule guide (green strip)

Using the transplant date in Patient Info, the sheet reads: "Post-Tx 5 m 9 d ▪ Bi-monthly visits (60 d)" and suggests a date. The standard intervals are:

Time since transplant	Visit every	Name
First month	15 days	Fortnightly
After 1 month, until 6 months	30 days	Monthly
After 6 months, until 1 year	60 days	Bi-monthly
After 1 year	90 days	Quarterly

If the usual interval would land the next visit in a later phase, the later (longer) interval is used — which matches how doctors normally relax the schedule. **Your doctor's date always overrides the suggestion.**

MISSING NUMBERS ARE LABELLED, NOT HIDDEN

"Not Avl" appears when no value exists for that date: for example, if the Daily Vitals Log has no entry that day, or no lab was done on or before it. Where an earlier result exists it is shown, in darker pink, so you are never left with an empty cell.

5. Lifecycle & Triggers

When	What to do
First day of the workbook	Enter the HEIGHT (once).
After each consultation (same day or the next)	Do the three-move routine; add one row.
Before the visit	Check the "Next Consultation Date" of the last row — is it today's visit?
Up to 50 visits per workbook	Rows 6 to 55. At the end, start a new workbook.

CHAPTER 10 · REFERENCE SHEET

Chapter 10: Lab Reference Ranges

The meaning of the numbers: general normal ranges and post-transplant targets.

1. Purpose & Requirement

Normal ranges for healthy people are often not the right targets for someone with a kidney transplant. **Lab Reference Ranges** lists, for each test, the general normal range, the **post-transplant target**, and a short note about why the test matters. It is a reading aid — the workbook's automatic alerts use the editable limits in Dashboard Settings (Chapter 11).

- **Metrics compiled:** about 28 laboratory tests plus vitals targets (blood pressure, SpO_2, temperature, BMI, urine output).
- **Seven groups:** renal function, immunosuppression, electrolytes, metabolic/glucose, haematology, vitals targets and other monitoring.
- **Static information** — it does not change when you enter data.

LABORATORY REFERENCE RANGES — KIDNEY TRANSPLANT PATIENTS

Use these ranges to interpret follow-up lab values. Values outside TX Target require medical review.

Parameter	Unit	Normal Range	Target (Post-Transplant)	Notes / Clinical Significance
RENAL FUNCTION				
Serum Creatinine	mg/dL	0.6 – 1.2	Stable baseline (patient-specific)	Rising trend may indicate rejection
eGFR	mL/min	> 60	> 45 (acceptable > 30)	CKD staging based on eGFR
Urine PCR (UACR)	mg/g	< 150	< 300 (stable graft)	Proteinuria indicates graft stress
24h Urine Protein	mg/24h	< 150	< 300	Elevated in nephrotic syndrome
IMMUNOSUPPRESSION				
Tacrolimus Trough	ng/mL	—	Early (0–3m): 8–12 | Late (>6m): 5–8	Adjust dose per trough level; toxicity >15
BKV PCR	copies/mL	Negative	< 10,000 (low viremia)	High viremia → BK nephropathy risk
CMV PCR	copies/mL	Negative	Undetectable	Monitor monthly × 6 months post-Tx
ELECTROLYTES				
Potassium	mEq/L	3.5 – 5.0	3.5 – 5.0	Hyperkalaemia with tacrolimus/CNIs
Sodium	mEq/L	135 – 145	135 – 145	—
Calcium (Total)	mg/dL	8.5 – 10.5	8.5 – 10.2	Hypercalcaemia → graft calcification risk
Phosphorus	mg/dL	2.5 – 4.5	2.5 – 4.5	Hypophosphataemia post-Tx
Serum Bicarbonate	mEq/L	22 – 29	22 – 29	Metabolic acidosis post-transplant
Uric Acid	mg/dL	2.4 – 7.0	< 7.0	Gout risk with cyclosporine
METABOLIC / GLUCOSE				
FBS (Fasting Sugar)	mg/dL	70 – 100	< 126 (target: avoid PTDM)	PTDM risk with tacrolimus
PPBS (Post-meal)	mg/dL	< 140	< 180	Post-Transplant Diabetes Mellitus
HbA1c	%	≤ 5.7	≤ 7.0	3-monthly monitoring
HAEMATOLOGY				
Haemoglobin (Hb)	g/dL	12 – 17	11 – 14	Anaemia common post-transplant
TLC (WBC)	$\times 10^3/\mu L$	4.0 – 11.0	4.0 – 10.0	Leukopenia may indicate drug toxicity
Platelets	$\times 10^3/\mu L$	150 – 400	150 – 350	Monitor for thrombocytopenia
Serum Albumin	g/dL	3.5 – 5.0	3.5 – 5.0	Low albumin = malnutrition / nephrotic
VITALS TARGETS				
Blood Pressure	mmHg	< 120/80	< 130/80	Strict BP control protects graft
SpO_2	%	≥ 95	≥ 94	Alert if < 94%
Temperature	°F	97 – 99	< 100	Fever (>100.4°F) = urgent review
BMI	kg/m^2	18.5 – 24.9	18.5 – 27	Obesity increases graft failure risk
Urine Output	mL/day	≥ 1000	≥ 1500 (preferred)	Oliguria = urgent review
OTHER MONITORING				
Vitamin D (25-OH)	ng/mL	30 – 100	30 – 100	Supplement if deficient
LDL Cholesterol	mg/dL	< 100	< 100	Cardiovascular risk management
SGOT / AST	U/L	≤ 40	≤ 40	Liver function — 3-monthly
SGPT / ALT	U/L	≤ 56	≤ 56	Liver function — 3-monthly

Figure 17 The Lab Reference Ranges sheet

Each row gives a parameter, its unit, the normal range, the post-transplant target and a note on clinical significance. For example, tacrolimus trough targets 8–12 ng/mL early and 5–8 ng/mL late after transplant.

2. Visual Layout Blueprint

Map of the Lab Reference Ranges sheet

```
┌──────────────────────────────────────────────────────────────────────────────────────────────┐
│ LABORATORY REFERENCE RANGES - KIDNEY TRANSPLANT PATIENTS                        [title bar]  │
│ Use these ranges to interpret follow-up lab values. Values outside TX Target require medical review. │
├──────────────────────────────────────────────────────────────────────────────────────────────┤
│ Parameter | Unit | Normal Range | Target (Post-Transplant) | Notes / Clinical Significance  │
├──────────────────────────────────────────────────────────────────────────────────────────────┤
│ RENAL FUNCTION       Serum Creatinine, eGFR, Urine PCR (UACR), 24 h Urine Protein             │
│ IMMUNOSUPPRESSION    Tacrolimus Trough, BKV PCR, CMV PCR                                     │
│ ELECTROLYTES         Potassium, Sodium, Calcium, Phosphorus, Serum Bicarbonate               │
│ METABOLIC / GLUCOSE  Uric Acid, FBS, PPBS, HbA1c                                             │
│ HAEMATOLOGY          Haemoglobin, TLC (WBC), Platelets, Serum Albumin                        │
│ VITALS TARGETS       Blood Pressure, SpO2, Temperature, BMI, Urine Output                    │
│ OTHER MONITORING     Vitamin D, LDL Cholesterol, SGOT/AST, SGPT/ALT                          │
└──────────────────────────────────────────────────────────────────────────────────────────────┘
```

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Navy	**Navy bars**	Group names — Renal Function, Electrolytes, Haematology, and so on.
Grey	**Grey table header**	Column titles.
Aa 123	**White rows**	Fixed reference text — no data entry.

4. Data Input Requirements

None. This sheet is for reading only. If your doctor sets a different target for you, record it in **Dashboard Settings** (Chapter 11), where the limits drive the alerts and the chart lines.

> **TARGETS CAN DIFFER BY MONTHS SINCE TRANSPLANT**
> Some targets change over time — for example tacrolimus is usually higher in the first months. The sheet shows both “early” and “late” bands. Always follow your own doctor’s current target.

5. Lifecycle & Triggers

When	What to do
When a lab result surprises you	Look up the test here to see what is normal and what it means.
When your doctor changes a target	Update Dashboard Settings (not this sheet).

CHAPTER 11 · SETTINGS SHEET · EDIT RARELY

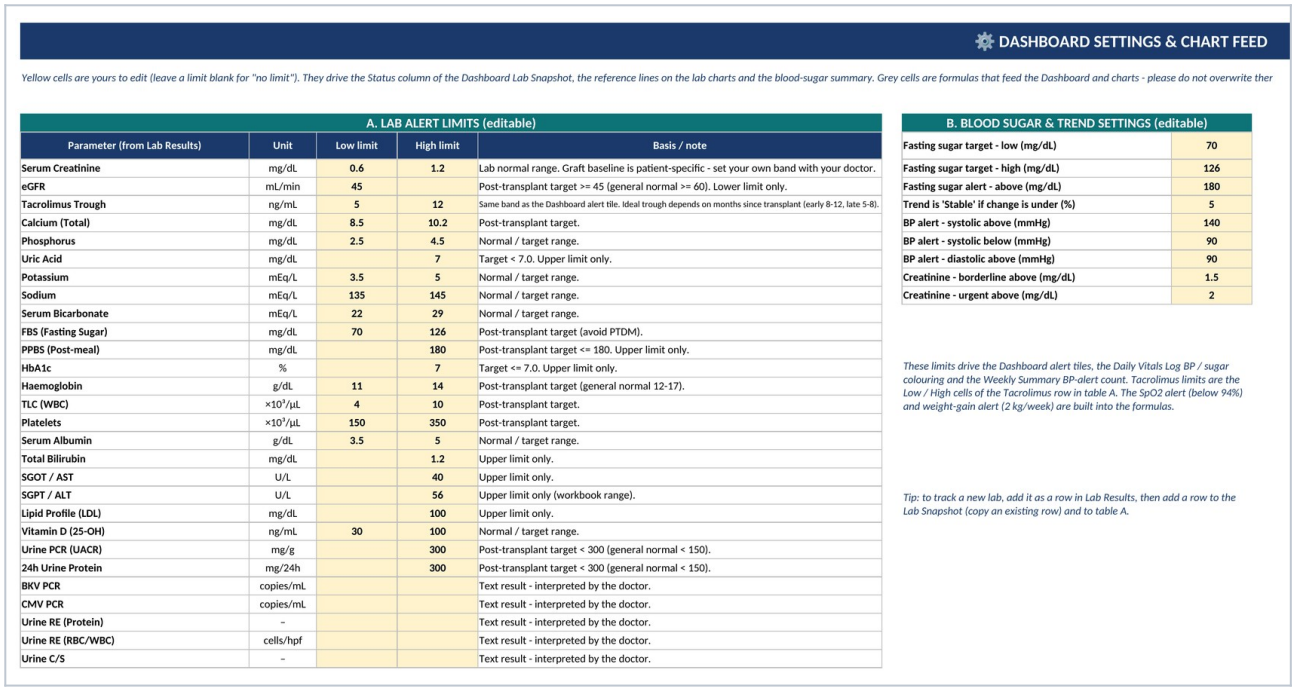
Chapter 11: Dashboard Settings

The control room: alert limits that colour your charts and tiles.

1. Purpose & Requirement

Dashboard Settings holds the limits that decide when something is “high”, “low” or “in range”. They drive the Status column of the Lab Snapshot, the limit lines on the lab charts, the blood-sugar summary, the red colouring in the Daily Vitals Log and the BP-alert count of the Weekly Summary.

- **Table A — Lab alert limits:** a low and a high limit for each lab test, with the reason for each default.
- **Table B — Blood sugar and trend settings:** sugar targets and alert line, the “stable trend” percentage, the BP alert limits and the creatinine warning levels.
- **Table C — Lab-chart reference lines:** formulas that draw the dashed limit lines. Do not edit.
- **Helper formulas:** grey columns that feed the Dashboard. Do not edit.



DASHBOARD SETTINGS & CHART FEED

Yellow cells are yours to edit (leave a limit blank for "no limit"). They drive the Status column of the Dashboard Lab Snapshot, the reference lines on the lab charts and the blood-sugar summary. Grey cells are formulas that feed the Dashboard and charts - please do not overwrite ther

A. LAB ALERT LIMITS (editable)

Parameter (from Lab Results)	Unit	Low limit	High limit	Basis / note
Serum Creatinine	mg/dL	0.6	1.2	Lab normal range. Graft baseline is patient-specific - set your own band with your doctor.
eGFR	mL/min	45		Post-transplant target >= 45 (general normal >= 60). Lower limit only.
Tacrolimus Trough	ng/mL	5	12	Same band as the Dashboard alert tile. Ideal trough depends on months since transplant (early 8-12, late 5-8).
Calcium (Total)	mg/dL	8.5	10.2	Post-transplant target.
Phosphorus	mg/dL	2.5	4.5	Normal / target range.
Uric Acid	mg/dL		7	Target < 7.0. Upper limit only.
Potassium	mEq/L	3.5	5	Normal / target range.
Sodium	mEq/L	135	145	Normal / target range.
Serum Bicarbonate	mEq/L	22	29	Normal / target range.
FBS (Fasting Sugar)	mg/dL	70	126	Post-transplant target (avoid PTDM).
PPBS (Post-meal)	mg/dL		180	Post-transplant target <= 180. Upper limit only.
HbA1c	%		7	Target <= 7.0. Upper limit only.
Haemoglobin	g/dL	11	14	Post-transplant target (general normal 12-17).
TLC (WBC)	$\times 10^3/\mu L$	4	10	Post-transplant target.
Platelets	$\times 10^3/\mu L$	150	350	Post-transplant target.
Serum Albumin	g/dL	3.5	5	Normal / target range.
Total Bilirubin	mg/dL		1.2	Upper limit only.
SGOT / AST	U/L		40	Upper limit only.
SGPT / ALT	U/L		56	Upper limit only (workbook range).
Lipid Profile (LDL)	mg/dL		100	Upper limit only.
Vitamin D (25-OH)	ng/mL	30	100	Normal / target range.
Urine PCR (UACR)	mg/g		300	Post-transplant target < 300 (general normal < 150).
24h Urine Protein	mg/24h		300	Post-transplant target < 300 (general normal < 150).
BKV PCR	copies/mL			Text result - interpreted by the doctor.
CMV PCR	copies/mL			Text result - interpreted by the doctor.
Urine RE (Protein)	-			Text result - interpreted by the doctor.
Urine RE (RBC/WBC)	cells/hpf			Text result - interpreted by the doctor.
Urine C/S	-			Text result - interpreted by the doctor.

B. BLOOD SUGAR & TREND SETTINGS (editable)

Fasting sugar target - low (mg/dL)	70
Fasting sugar target - high (mg/dL)	126
Fasting sugar alert - above (mg/dL)	180
Trend is 'Stable' if change is under (%)	5
BP alert - systolic above (mmHg)	140
BP alert - systolic below (mmHg)	90
BP alert - diastolic above (mmHg)	90
Creatinine - borderline above (mg/dL)	1.5
Creatinine - urgent above (mg/dL)	2

These limits drive the Dashboard alert tiles, the Daily Vitals Log BP / sugar colouring and the Weekly Summary BP-alert count. Tacrolimus limits are the Low / High cells of the Tacrolimus row in table A. The SpO2 alert (below 94%) and weight-gain alert (2 kg/week) are built into the formulas.

Tip: to track a new lab, add it as a row in Lab Results, then add a row to the Lab Snapshot (copy an existing row) and to table A.

Figure 18 Dashboard Settings (editable yellow cells, grey formula feed)

Left: Table A lists each test with its unit and low/high limits plus the basis for the default. Middle: Table B (sugar, trend, BP and creatinine settings). Right: grey helper columns that feed the Dashboard — not for editing.

2. Visual Layout Blueprint

Map of the Dashboard Settings sheet

```
┌──────────────────────────────────────────────────┬──────────────────────────────────────────┐
│ DASHBOARD SETTINGS & CHART FEED  (title)         │ Yellow = yours to edit; grey = formulas  │
├──────────────────────────────────────────────────┼──────────────────────────────────────────┤
│ A. LAB ALERT LIMITS (editable)                   │ B. BLOOD SUGAR & TREND SETTINGS (editable)│
│ Parameter | Unit | Low limit | High limit | Basis │   Fasting sugar target - low / high       │
│ Serum Creatinine  mg/dL  0.6   1.2   ...         │   Fasting sugar alert - above (mg/dL)    │
│ eGFR              mL/min 45    --    ...         │   Trend is "Stable" if change under (%)  │
│ Tacrolimus Trough ng/mL  5     12    ...         │   BP alert - systolic above / below      │
│ Calcium, Phosphorus, Uric Acid ... CMV           │   BP alert - diastolic above             │
│ (27 rows: one per lab test)                      │   Creatinine - borderline / urgent above │
├──────────────────────────────────────────────────┼──────────────────────────────────────────┤
│ Helper formulas (grey - do not edit):            │ C. LAB-CHART REFERENCE LINES (grey feed) │
│ latest / previous column, running list           │   Lower / upper limit lines drawn on charts│
└──────────────────────────────────────────────────┴──────────────────────────────────────────┘
```

3. Color-Coded Guidance

Colour	What you see	What it means / what to do
Aa 123	**Pale-yellow cell**	**Yours to edit.** Leave a limit blank for “no limit”.
Aa 123	**Grey cell**	A formula that feeds the Dashboard and charts. **Do not overwrite.**
Navy	**Navy headers**	Table titles.

4. Data Input Requirements

Enter **plain numbers** only (a decimal point where needed). Words or negative numbers are refused with an error message. The default values are:

Test (Table A)	Unit	Low limit	High limit	Basis
Serum Creatinine	mg/dL	0.6	1.2	Lab normal; graft baseline is personal
eGFR	mL/min	45	—	Post-transplant target ≥ 45
Tacrolimus Trough	ng/mL	5	12	Same band as the Dashboard tile
Calcium (Total)	mg/dL	8.5	10.2	Post-transplant target
Phosphorus	mg/dL	2.5	4.5	Normal / target
Uric Acid	mg/dL	—	7	Target < 7.0
Potassium	mEq/L	3.5	5	Normal / target
Sodium	mEq/L	135	145	Normal / target
Serum Bicarbonate	mEq/L	22	29	Normal / target
FBS (Fasting Sugar)	mg/dL	70	126	Avoid PTDM
PPBS (Post-meal)	mg/dL	—	180	Target ≤ 180
HbA1c	%	—	7	Target ≤ 7.0
Haemoglobin	g/dL	11	14	Post-transplant target
TLC (WBC)	$\times 10^3/\mu L$	4	10	Post-transplant target
Platelets	$\times 10^3/\mu L$	150	350	Post-transplant target

Test (Table A)	Unit	Low limit	High limit	Basis
Serum Albumin	g/dL	3.5	5	Normal / target
Total Bilirubin	mg/dL	—	1.2	Upper limit only
SGOT / AST	U/L	—	40	Upper limit only
SGPT / ALT	U/L	—	56	Upper limit only
Lipid Profile (LDL)	mg/dL	—	100	Upper limit only
Vitamin D (25-OH)	ng/mL	30	100	Normal / target
Urine PCR (UACR)	mg/g	—	300	Target < 300
24h Urine Protein	mg/24h	—	300	Target < 300

Table B defaults: fasting sugar target low **70** and high **126**; fasting sugar alert above **180** mg/dL; a trend is called "Stable" if the change is under **5 %**; BP alert — systolic above **140**, systolic below **90**, diastolic above **90**; creatinine borderline above **1.5** and urgent above **2.0** mg/dL.

CHANGE LIMITS ONLY WITH YOUR DOCTOR

Moving a limit changes the colours and messages you see, not your health. A limit that is too wide can hide a real problem. Note the date and the reason whenever you change one.

TWO BUILT-IN ALERTS CANNOT BE EDITED HERE

The SpO_2 alert (**below 94 %**) and the weight-gain alert (**2 kg in a week**) are built into the formulas. Tacrolimus limits are the Low/High cells of the tacrolimus row in Table A.

5. Lifecycle & Triggers

When	What to do
First day	Review the defaults with your doctor; change only what is needed.
Your doctor changes a target	Update the matching limit(s) and note the date.
New quarterly workbook	Keep the same limits — they carry over.
Never	Do not type in grey cells or in Table C / helper columns.

Clinical Consultation & Follow-Up Directive

This is the most important section of the manual. Read it before **every** scheduled consultation. It tells you exactly what to send, and how to be sure the numbers are right.

BEFORE EVERY SCHEDULED CONSULTATION

MUST be printed or e-mailed to the physician: the three ★ sheets — **Doctor Referral Report** (1 page), **Dashboard** (2 pages) and **Daily Vitals Log** (about 3 pages). Together they are about **six pages**.

Keep ready, but send only if the doctor asks: Lab Results, Medication History, Weekly Summary (1 page) and the **original hospital lab reports**.

Do not send the entire workbook. It adds 8+ pages of raw tables, buries the key findings and shares more personal data than needed.

The Doctor Pack at a glance

Sheet	Pages	What the doctor learns
Doctor Referral Report ★	1	Who you are, last-7-day vitals, latest labs, current medicines, and space for the plan.
Dashboard ★	2	Automatic alerts, creatinine / eGFR / tacrolimus trends, lab snapshot, BP, weight, urine, SpO_2 and sugar trends.
Daily Vitals Log ★	about 3	Every day's detail, for the doctor to study when a trend needs explaining.

Checklist: pre-appointment data verification

Tick each box. Do this **two days before** the visit so that you have time to correct anything.

A. Daily Vitals Log

- ☐ The last row is **yesterday or today** — no missing days.
- ☐ Every row has a date, BP (both numbers), pulse, SpO_2, weight and fluid in.
- ☐ No "0" has been typed where a reading was simply not taken (those cells should be empty).
- ☐ Any red or ⚠️ flag has a note explaining it (for example "felt dizzy", "re-measured: 132/80").
- ☐ Blood sugar readings are **fasting** values, taken before breakfast.

B. Lab Results

- ☐ The **newest** lab report is entered, in the next free column, with its test date.
- ☐ Dates run **oldest → newest**, with no gap between columns.
- ☐ Units match column C (for example platelets $\times 10^3/\mu L$; white cells in thousands).
- ☐ Values that were not tested are left blank — not zero.

C. Medication History

- ☐ Every medicine you **take now** has Status = **ACTIVE**.
- ☐ Old doses are marked **CHANGED** (with the change date); stopped medicines are **STOPPED** (with the stop date).
- ☐ **Last Prescription Date** is updated.
- ☐ The Referral Report's medicine list matches your pill box and the latest prescription.

D. The three Doctor Pack sheets

- ☐ **Dashboard:** read the alert tiles. Anything marked ⚠️ or 🚨 — write it down as a question for the doctor.

- ☐ **Referral Report:** the identity block, medicines and latest labs are correct. The "Notes / Plan / Lab Tests" lines are empty and ready.
- ☐ The Next Visit Date dropdowns read **DD / MMM / YYYY** (reset from the last visit).
- ☐ **Doctor Visit Log:** the last row is the previous visit and its "Next Consultation Date" is today's visit.
- ☐ You have saved a **dated copy** of the workbook (for example `Nephro_Tracker_05Oct2026.xlsx`).
- ☐ The PDF opens correctly and contains **only the three sheets** (about six pages).

✓ **DONE WHEN...**
Every box is ticked, the PDF is saved with a clear name, and you have sent it to your doctor's e-mail or WhatsApp **before** the visit — or printed it and placed it in your hospital file.

On the day of the visit

- Carry **printed copies** of the Doctor Pack and the **original hospital lab reports**.
- Have your medicine strips or pill box with you.
- Bring your list of questions (see the Reader's Notes at the end of this manual).
- Ask the doctor to write the **plan, the tests required and the next visit date** on the Referral Report, or note them down yourself.

After the visit — close the loop

1. Enter the doctor's notes, plan, tests required and **Next Visit Date** on the Referral Report.
2. Update **Medication History** for every new, changed or stopped medicine.
3. Add one row to the **Doctor Visit Log** (pick the date, copy, paste values, fill the white columns).
4. Set a phone reminder for the lab tests and the next appointment.
5. Reset the Referral Report's plan area for the next cycle when you start the next Doctor Pack.

Errata: Common Pitfalls and How to Fix Them

Everyone makes data-entry mistakes. The workbook is built to be forgiving, and most problems have a simple cure. Use this section as a troubleshooting guide.

Spotting formula errors

A formula error is a short message beginning with `#` that appears instead of a number. It is a sign that something the formula depends on is wrong or missing.

You see	What it usually means	How to resolve it
`#VALUE!`	A cell holds text where a number is expected (for example "12 mg" or "N/A" typed into a numeric cell).	Find the cell the formula reads from; replace the text with a plain number, or leave the cell empty.
`#REF!`	A row or column that a formula depends on was **deleted** or a sheet was removed.	Press `Ctrl+Z` immediately to undo. If it was saved, reopen the last dated copy. Never delete rows, columns or sheets.
`#NAME?`	Excel does not recognise a name or function — usually an old Excel version meeting a newer function, or a sheet that was renamed.	Update to Excel 2013 or newer; never rename the sheets. If a figure shows `#NAME?` in the Referral Report's Min/Max columns, use the corrected workbook (v13 or later).
`#DIV/0!`	A calculation divides by zero or by an empty cell (for example BMI with no height).	Enter the missing value — for BMI, the **HEIGHT** in the Doctor Visit Log.
`#N/A`	A lookup did not find a match.	Check that dates match exactly and that you typed real dates (not text).
`#####`	The column is too narrow for the number or date.	Widen the column by dragging its right edge — or reduce Zoom.

Data-entry pitfalls

Pitfall	Why it happens and what to do
Dates refused or charts empty	Dates typed as text (for example "5th Oct") or typed out of order. Use `dd-mmm-yyyy`; in Lab Results enter dates oldest first with no gap.
Date shows as 05-10-2026 but means the wrong day	Typing numbers only is read according to your computer's regional setting (day-month vs month-day). Prefer `05-Oct-2026`, or pick from the dropdown.
A zero where a reading was missed	A typed `0` is treated as a real reading and drags the averages down. Leave the cell empty.
Overwriting a grey / white cell	You may have typed over a formula. Press `Ctrl+Z`; or copy the same cell from the last dated backup.
Normal paste in the Doctor Visit Log	The row keeps changing when you pick another date. Re-paste with **Paste Special ▸ Values**; cells turn orange as a warning.
Pasting "Not Avl" into a number column	It is only a label. Replace it with the real value (or leave it as a record that nothing was available).
Three sheets left selected after printing	Your next typing goes into all three. Click any other tab to un-select them.
HEIGHT changed midway	Earlier BMI values were calculated with the old height. Enter height once and keep it.

Pitfall	Why it happens and what to do
More than 15 active medicines	Only 15 fit on the Referral Report; the heading shows how many are not shown. The full list stays in Medication History.
Platelets entered as 2.79	The unit is $\times 10^3/\mu L$; enter `279`.

Things that look odd but are normal

- **Pink empty cells** in the BP Systolic and SpO_2 columns of the Daily Vitals Log — an empty cell is treated as "too low". They turn normal when you type a value.
- **The first two rows of the Fluid In column** may have a slightly different colour from later rows (the colour scale starts separately). It is cosmetic.
- **The word "Calculate"** in the Excel status bar means Excel is in manual-calculation mode (a setting from another open workbook). Press `F9`, or choose **Formulas ▸ Calculation Options ▸ Automatic**.
- **A dropdown arrow that does not appear** next to a date cell after you scroll sideways — scroll back (`Ctrl+Home`) and click the cell again. Typing the date always works.
- **The group titles in Lab Results** (for example, tacrolimus under "Renal Function") are only headings; the calculations are correct for each test.
- **The green / yellow / red legend in the Lab Results header** is a reading guide. The automatic Normal / High / Low judgement is shown on the Dashboard's Lab Snapshot.
- **The "Date" column in the Lab Snapshot** may be older than the latest column: each test shows when **it** was last done.

If something is badly broken

1. Close the file **without saving**, and reopen your latest dated copy.
2. If the problem remains, open the original file you saved on Day 1.
3. Re-enter the missing days from your paper notes or from the Doctor Pack PDFs you e-mailed.



THE BEST PROTECTION IS A HABIT

Save a **dated copy** every Sunday and before each doctor's visit, for example `Nephro_Tracker_05Oct2026.xlsx`. Keep a second copy on a pen drive or in your e-mail.

Reader's Notes — Questions for My Doctor

Write your questions and anything unusual here **before** the appointment, so that nothing is forgotten. Note the doctor's answer during the visit. Copy this page as often as you need.

Visit date: __ / / **Doctor:** _____

No.	My question or something I noticed	Doctor's answer / advice	Done ✔
1			
2			
3			
4			
5			
6			
7			
8			
9			
10			

Symptoms and readings to mention

Tick what applies and add the date it started.

- ☐ Higher blood pressure than usual — since: _____
- ☐ Weight gain or swelling of feet / face — since: _____
- ☐ Less urine than usual, or burning while passing urine — since: _____
- ☐ Fever, cough, breathlessness or low SpO_2 — since: _____
- ☐ Very high or very low blood sugar — since: _____
- ☐ Missed doses, side effects or new tablets from another doctor — since: _____

Page 2: Plan and Next Steps

Heading	My notes
Medicines changed	
Tests to do before the next visit	
Diet / exercise advice	
Next visit date	
Other advice	

Questions that came up later

No.	My question or something I noticed	Doctor's answer / advice	Done ✔
1			
2			
3			
4			
5			
6			
7			
8			



Small numbers, written down every day, make big conversations possible.

Keep this manual beside your tracker. Record your readings, update your labs and medicines, and send the **Doctor Pack** before every visit — so that every minute with your doctor is spent on **you**.

Important: the tracker supports, but never replaces, medical advice. Values in **red or orange** mean "inform your doctor". Never change a medicine on your own.

DESIGN NOTE

Back cover composed in the workbook's own palette — clinical teal fading to deep navy, with a warm amber accent — and flat, friendly symbols of care: a protective shield, a hospital, medicine, a stethoscope and the calendar of follow-up visits.